



2023 - 2030 NATIONAL STRATEGIC PLAN **FOR HIV**





2023 - 2030
NATIONAL
STRATEGIC
PLAN FOR HIV

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ABBREVIATIONS AND ACRONYMS

A&E	Accident and Emergency
AGYW	Adolescent Girls and Young Women
AIDS	Acquired Immunodeficiency Syndrome
ALHIV	Adolescent Living with HIV
ANC	Antenatal Care
ART	Antiretroviral Treatment
ARV	Antiretroviral
BCC	Behaviour Change Communication
CARICOM	The Caribbean Community
CBO	Community Based Organisation
CCH	The Caribbean Cooperation in Health
CI	Contact Investigator
CLM	Community-Led Monitoring
CSO	Civil Society Organisations
EEHR	Enabling Environment and Human Rights
EID	Early Infant Diagnosis
EMTCT	Elimination of Mother-to-Child Transmission
FBO	Faith Based Organisation
FP	Family Planning
FPSD	Family Planning Services Division
FSW	Female Sex Worker
GBV	Gender Based Violence
GDP	Gross Domestic Product
GFATM	Global Fund for AIDS, Tuberculosis and Malaria
GOJ	Government of Jamaica
HC	Health Centre
HCW	Health Care Worker
HDI	Human Development Index
HIV	Human Immunodeficiency Virus
HIVDR	HIV Drug Resistance
HPE	Health Promotion and Education
HSTU	HIV/STI/TB Unit
IEC	Information, Education & Communication
JaBCHA	Jamaica Business Council on HIV and AIDS
JASL	Jamaica AIDS Support for Life
JCCM	Jamaica Country Coordinating Mechanism
JFLAG	Jamaica Forum for Lesbians, All-Sexuals and Gays
KAPB	Knowledge, Attitude, Behaviour and Practice
LCA	Life Course Approach
LTFU	Lost to Follow-Up
M&E	Monitoring and Evaluation
MCGES	Ministry of Culture, Gender, Entertainment & Sports
MDAs	Ministries, Departments and Agencies
MERG	Monitoring and Evaluation Reference Group

MLSS	Ministry of Labour and Social Security
MoEY	Ministry of Education, Skills, Youth & Information
MOHW	Ministry of Health and Wellness
MOJ	Ministry of Justice
MOU	Memorandum of Understanding
MSM	Men Who Have Sex with Men
NAC	National AIDS Committee
NFPB	National Family Planning Board
NGO	Non-Government Organization
NHF	National Health Fund
NHP	National HIV/STI Programme
NHRI	National Human Rights Institution
NISP	National Integrated Strategic Plan for Sexual and Reproductive Health & HIV
NLS	National Laboratory Services
NPHL	National Public Health Laboratory
NSP	National Strategic Plan
NSU	National Surveillance Unit
PANCAP	Pan Caribbean Partnership Against HIV and AIDS
PCR	Polymerase Chain Reaction
PEPFAR	U.S. President's Emergency Plan for AIDS Relief
PHC	Primary Health Care
PHDP	Positive Health Dignity Prevention
PIOJ	Planning Institute of Jamaica
PITC	Provider Initiated Testing and Counselling
PLHIV	People Living with HIV
PMTCT	Prevention of Mother-to-Child Transmission
PrEP	Pre-Exposure Prophylaxis
RHA	Regional Health Authority
S&D	Stigma and Discrimination
SDGs	Sustainable Development Goals
SERHA	South East Regional Health Authority
SMS	Short Message Service
SRH	Sexual and Reproductive Health
STI	Sexually Transmitted Infection
STATIN	Statistical Institute of Jamaica
SOP	Standard Operating Procedures
SW	Social Worker
TSIS	Treatment Services Information System
TWG	Technical Working Group
UDHR	The Universal Declaration of Human Rights
UHC	Universal Health Coverage
UNAIDS	The Joint United Nations Programme on HIV/AIDS
UNICEF	The United Nations Children's Fund
UNDP	United Nations Development Programme
VL	Viral Load
WHO	World Health Organization
WLHIV	Women Living with HIV

INTRODUCTION

Jamaica has taken a proactive approach to controlling the HIV epidemic through the establishment of a multi-sectoral response led by the Ministry of Health and Wellness (MOHW). The number of new HIV infections has declined by 62% between 2004 and 2021, and 84% of people living with HIV (PLHIV) have been diagnosed. Additionally, HIV prevalence has decreased among sexually transmitted infection (STI) clinic attendees, pregnant women and female sex workers. Despite these achievements, at the end of 2021, National Surveillance Unit (NSU) & Treatment Services Information System (TSIS) data reflect that there were approximately 350 deaths due to HIV and AIDS on an annual basis. An estimated 1400 new cases in 2021, of which 691 were diagnosed, 2854 persons are still unaware of their HIV status, 3,446 persons have defaulted from care and 3,661 persons on Antiretroviral Treatment (ART) are not virally suppressed (MOHW NSU data and Programmatic data). In addition, the HIV prevalence among men who have sex with men (MSM) has remained high for nearly three decades, ranging from 29.6% to 31% (876 2018 Study). Stigma and discrimination associated with HIV and stigma aimed at key populations remain a challenge within the Jamaican context. In addition, culture, legislative and policy framework also continue to threaten the control of the HIV epidemic. Adolescents and youth (15-24) accounted for 16% of new HIV infections in 2021. Young people remain a vulnerable group based on documented evidence of high-risk behaviours; 47% of in-school adolescents (13-17 years) are sexually active, with 61% having their first sexual encounter before the age of 14 (Global School Health Survey 2017). In addition, young people (15-24) account for the highest proportion of cases of sexually transmitted infections (STIs) reported in 2021.

Under the leadership of the Government, Jamaica will continue to implement a multi-sectoral response to HIV with involvement of Government Ministries, Departments and Agencies (MDAs), civil society, private sector, churches and persons living with HIV. Jamaica has also benefited from ongoing financial support and technical assistance from several international, regional and bilateral partners.

Over the years, numerous efforts have been made to establish a suitable governance structure to sustain the HIV response, which is heavily donor funded. Accordingly, since 2016, the MOHW has consistently taken steps to absorb the HIV-related staff costs previously funded by the project. In this

strategic period, additional steps will be taken in this regard. The re-establishment of the National AIDS Committee and the integration of the leadership of the MOHW HIV Programme into a single unit are key actions.

Recommendations emanating from Jamaica's Sustainability and Transition Plan include strengthening the coordination of the national HIV response by appointing an appropriately empowered and staffed national AIDS council to achieve 'Sustainable Governance and Enabling Human Rights Environment for Sustainability'. The proposed revitalization of the National AIDS Committee should help to improve sustainability and strengthen continued representation from communities following the transition from donor funding. In keeping with the MOHW public-private partnership model outlined in the Primary Health Care Renewal Plan and the Ministry's Ten-Year Development Plan, the coordination of the multi-sectoral response, including private sector and civil society organizations, will remain an engine for development of the health sector over the next decade.

Integration of HIV services into public health systems, including at the national and sub-national levels, is crucial for the sustainability of the NHP and will not only positively impact HIV Prevention and Treatment but will improve overall programme outcomes as well. The re-integration of the MOHW HIV Programme leadership will improve programme coordination. Integration with the National Health Fund should improve supply chain management.

At the service delivery level, the integration of the HIV Prevention team into the Health Promotion and Education team, full integration of HIV treatment services into primary health care, decentralization of the laboratory services, along with the opportunities presented by the primary health care renewal strategies, augur well for a strengthened HIV Response. The upgrading of public health centres, inclusion of private providers in the service delivery network, establishment of a Health Information System and the integrated health service delivery model will improve access to, and the quality of, HIV services.

Decentralization of National Laboratory Services (NLS) is the key strategy to support improving the effectiveness and efficiency of the laboratory services. Addressing lab capacity at the regional and sub-regional levels will ensure data and information

are available for the diagnosis and monitoring of patients as the country works towards achieving community viral suppression and epidemic control.

The MOHW has developed a Primary Health Care Reform 10-year plan (2021-2030) which outlines the vision for strengthening the delivery of population-based and individual care services through public and private facilities in the 13 health departments and 63 health districts that characterize Jamaica's health system. As part of the Ministry's efforts to achieve universal health care, the plan promotes a shift from disease centric to patient centric care, and concrete actions to redirect treatment services to maintain a renewed focus on the full spectrum of management of the patient from health promotion and prevention to diagnosis and care of specific illnesses. The strategies to achieve this entail improving the existing system for integration of health services, the seamless movement of patients, continuity of care and sharing of information, as well as developing mechanisms to coordinate health care services in the public and private sectors.

Specifically, the strategies outlined in Jamaica's primary health care reform model, which are a part of wider health system strengthening efforts that will boost the effectiveness of the HIV response, are as follows:

- An integrated health service delivery model - Vertical and horizontal integration of services linked through information systems management that supports coordination and seamless transitions across the continuum of care, including prevention, promotion, treatment, rehabilitation, and palliation. Continuity of care will also be prioritized through home health care and self-care to enhance self-management and adherence.
- Inclusion of private providers in the service delivery network - This public/private partnership

will allow for private primary care physicians to be engaged, thus making a comprehensive set of services more widely accessible to the population.

- Upgrading of public health centres and the establishment of multidisciplinary teams to provide more comprehensive patient-centred services, including specialists from different disciplines who will provide health services at the primary level. Specialized services will be delivered at the most appropriate location and not be confined to hospitals. The utilization of specialists from hospitals to support more resolute care at the primary level will be a part of this integrated service.
- Establishment of a Health Information System that will facilitate access to patient information and appointment systems at different sites will enable the seamless flow of patients from one service delivery site to the next. Provider to provider communication for better patient management and continued medical education, provider to patient connection for improved continuity of care and decrease in overcrowding and long waiting times, facility to facility connection for coordination of health services and improved access to information and facility to coordinating and monitoring site for timely data capture and analysis are all areas that will require technological solutions that will be the supporting framework for the integration of services

The Primary Health Care reform model being undertaken will allow for integrated, effective care in a continuum. The basic set of sexual and reproductive health (SRH) services offered at each life stage, as indicated in the PHC Plan, is outlined in Table 1.

Table 1: Basic Set of Sexual and Reproductive Health Services offered at each life stage

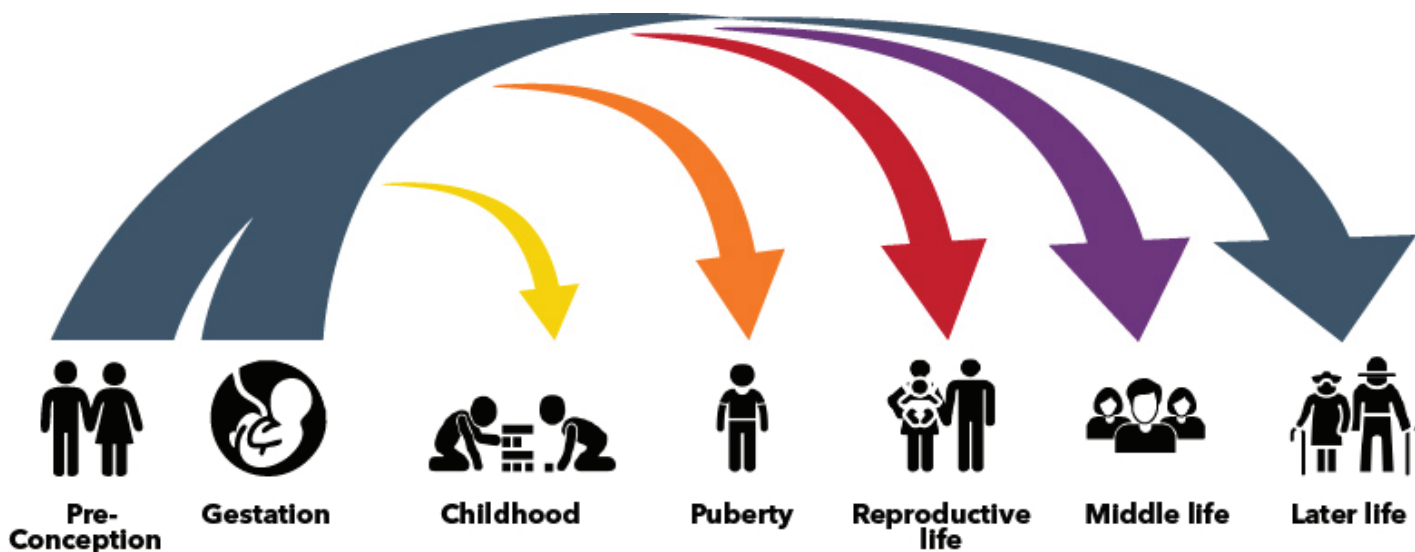
Pregnant Mother	Infant and Child	Adolescent	Adulthood	Elderly
Counselling and Education, Family Planning, STI/ HIV Management, Risk Assessment, PMTCT	Counselling and Education as indicated, appropriate Interventions	Counselling and Education, Family Planning, STI/ HIV Management, Risk Assessment, Prevention of Sexual Abuse and Violence	Family Planning, STI/HIV Management, Risk Assessment, Menopause and Andropause Management	STI/HIV Management, Risk Assessment, Menopause and Andropause management

Source: Primary Health Care Reform in Jamaica

[1] WHO and UNICEF. A vision for primary health care in the 21st century: towards universal health coverage and the Sustainable Development Goals. Geneva: World Health Organization and the United Nations Children's Fund (UNICEF), 2018 (WHO/HIS/SDS/2018.X). Licence: CC BY-NC-SA 3.0 IGO.

The integration of the Life Course Approach (LCA) in service delivery is a key strategy in the primary health care renewal thrust to improve health outcomes. The LCA aims to optimize health trajectories, connecting healthy development in the first decades of life with better health in middle and old age (Figure 1).

It means re-organizing health systems to focus on person-centred goals that not only manage disease, but also produce good health. This brings multiple benefits, including reducing inequalities and increasing people's ability to contribute to society.



Adapted from National Academies of Sciences, Engineering and Medicine, 2022

Figure 1: Life Course Approach Stages

Utilization of the LCA allows for the design of age-appropriate HIV-related interventions (education, risk-assessment, condom skills, screening, prophylaxis, etc.) to reduce the risk of HIV acquisition. These interventions will be integrated with other age-appropriate interventions to form holistic packages of care to be delivered to persons who access care in the various service delivery settings.

The successful implementation of this National HIV Strategic Plan requires the sustained effort of all stakeholders to build on the strengths of

the existing comprehensive programme and to address the weaknesses in the response. Successful implementation of the HIV National Strategic Plan will contribute to achieving Vision for Health 2030, Vision 2030 Jamaica and the Sustainable Development Goals (SDGs).

A monitoring and evaluation plan (attachment 1) has been developed for the NSP, and the estimated cost to implement the NSP is USD\$144,524,166 (attachment 2).

BACKGROUND

COUNTRY CONTEXT

Jamaica is an island country, 10,990 square kilometres (4,240 sq. mi), situated in the Caribbean Sea with a population of 2,734,092 (The Statistical Institute of Jamaica - STATIN 2019), the majority of whom are of African ancestry. The island has 14 parishes. The capital city, Kingston and the city of Montego Bay are two main urban centres. Population density is highest in the cities and urbanized parishes such as St. Catherine. Jamaica is a parliamentary democracy and a constitutional monarchy, achieving independence from Britain in 1962. It is an upper middle-income country (Gross Domestic Product - GDP per capita of US\$4,665) with an economy heavily dependent on tourism. The Gini index is 45.5, and the Human Development Index (HDI) is 0.73, ranking 101 out of 189 countries. Life expectancy is 76 years (74 for men and 77 for women); infant mortality is 12.4 per 1000 live births, and the fertility rate is 2.01 births per woman (Ministry of Health and Wellness, 2018). Jamaica has a well-developed public health system, which currently offers services with no user fees.

Governance of the National HIV Programme

The MOHW is mandated by the Government of Jamaica to coordinate and lead the implementation of the national HIV response. From the outset, the Government has taken a comprehensive, multi-sectoral approach to controlling the HIV/AIDS epidemic in Jamaica. The National HIV/STI Control programme was established in the Ministry of Health, and a National AIDS Committee (NAC) was formed to ensure the active participation of all sectors and stakeholders. Representation on the NAC included Government Ministries and Agencies, NGOs, professional associations, trade unions, faith-based organizations, PLHIV and the private sector. Currently, the NAC does not function in this role, and the Jamaica Country Coordinating Mechanism (JCCM) has assumed a coordinating role for the grantees under the Global Fund for AIDS, Tuberculosis and Malaria (GFATM) grant to the country.

Since 1988, Jamaica has had a national plan to guide the response to HIV and a well-established National HIV/STI Programme. A national HIV policy was accepted by Parliament in 2005, and the Government developed HIV policies for the

Education, Labour and Tourism sectors. In 1986, HIV and AIDS were made class one notifiable diseases. A governing body for the HIV response will be reintroduced during the tenure of this National Strategic Plan (NSP). The overall governance structure will be aligned as outlined in the diagram below. The previous National AIDS Committee will be reviewed, and steps taken to align with the requirements of the present HIV response.

The NGO sector has been integral in all aspects of the response, ranging from policy formulation and advocacy to service delivery. PLHIV-led organizations provide a channel for the PLHIV's voice in all aspects of the response. Faith-based organizations offer care and social support for the PLHIV, and some churches have developed HIV policies. The private sector has also played a role in the HIV response by working closely with the Government to develop HIV policies and programmes for the workplace in the early years through the Jamaica Business Council on HIV and AIDS (JaBCHA).

The Regional Health Authorities (RHAs) are responsible for service delivery for both the prevention and treatment arms of the response. Prevention interventions are implemented at the community level and among high-risk groups, while treatment is delivered in 39 health facilities (not inclusive of satellite sites). These 39 treatment sites have been integrated into the primary health care system to provide care to PLHIV across the country, with a concurrent strengthening of the STI care and treatment programmes.

Previous Strategic Plan

Jamaica's National Integrated Strategic Plan for Sexual and Reproductive Health and HIV (NISPH) 2014-2019 aimed to direct implementation of an integrated approach to Sexual and Reproductive Health (SRH) and HIV planning and programming. This was the first integrated Strategic Plan. In April 2013, the Jamaican Government aimed to integrate the National Family Planning Board (NFPB) and the National HIV/STI Programme out of financial necessity and a need to ensure sustainability of HIV and SRH responses. Further, the merger aimed to provide efficiencies from consolidating relevant entities with similar/overlapping mandates, who also had a sound track record providing dedicated services specific to HIV, AIDS, and SRH, with a focus on family planning.

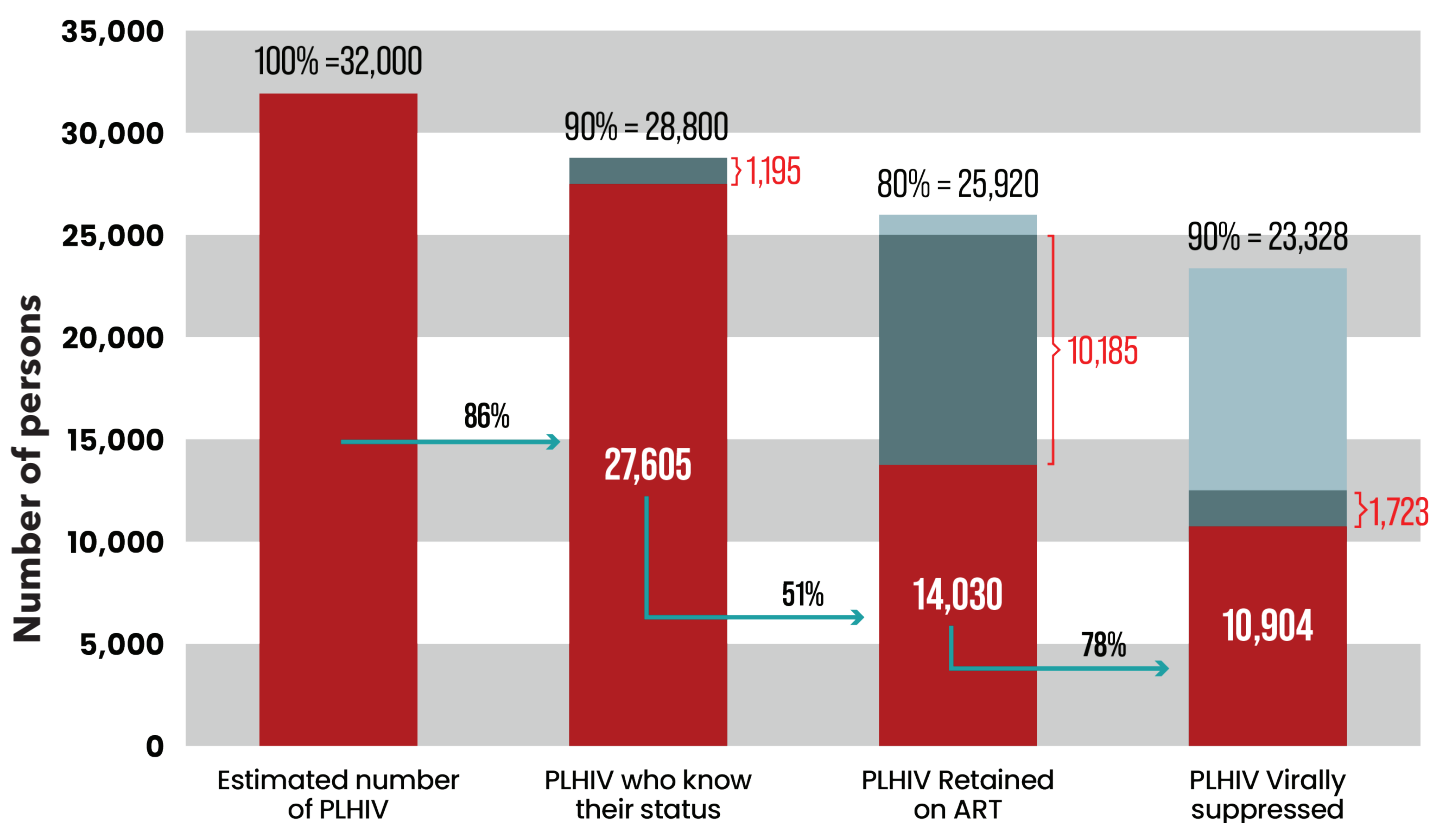
The NISP was structured around six overarching strategic outcomes:

1. Improved access to quality family planning services.
2. Reduced incidence of new HIV infections.
3. Improved access to quality treatment, care and support to reduce (SRH)/HIV-associated deaths.
4. Strengthened policy and legal framework for sexual and reproductive health and HIV prevention, treatment and care services.
5. Strengthened multi-sectoral partnerships to effectively plan, implement, monitor and evaluate programmes within an integrated SRH/HIV framework.
6. Enhanced capacity for the provision of integrated SRH and HIV services within the health sector.

A Mid-Term Evaluation was conducted in November 2017 and sought to assess the progress of Jamaica's HIV response by evaluating achievement of the NISP (2014 - 2019) objectives. The Mid-Term Evaluation found that while implementation is on track as it relates to achievement of some NISP outputs, the HIV-related NISP Targets (Outcomes) outlined above were not yet achieved.

EPIDEMIOLOGIC PROFILE

The adult HIV prevalence stands at 1.4%, with an estimated 32,000 PLHIV (The Joint United Nations Programme on HIV/AIDS - UNAIDS estimates, 2021). Jamaica's current achievement of the UNAIDS Fast Track Targets stands at 86-51-78 (Figure 2).



Source: Ministry of Health and Wellness Strategic Information Unit, 2019.

Figure 2: National HIV Cascade as at December 2021

Figure 2 identifies the three main areas that affect HIV epidemic control. Control will occur when HIV clients are diagnosed, maintained on medication and reach viral suppression. Jamaica did not attain the UNAIDS 90-90-90 targets for 2020, which called on Jamaica to diagnose 90% of its PLHIV population, place 90% of those diagnosed on treatment and ensure that 90% of those on treatment are virally suppressed. As at December 2021, Jamaica recorded gaps of 4% with respect to PLHIV aware of their status, 37% in treatment coverage and a 12%

gap for viral suppression. Even though the country has not achieved these targets, the NSP 2023-2030 speaks to attaining the 90-90-90 targets by 2025 and epidemic control by 2030.

Since the initial case of HIV was identified in Jamaica in 1982, 40,065 cases (52% males, 48% females) have been reported to the MOHW, and 13,077 deaths (59% males, 41% females) have been reported (as at December 2021) (Figure 3).

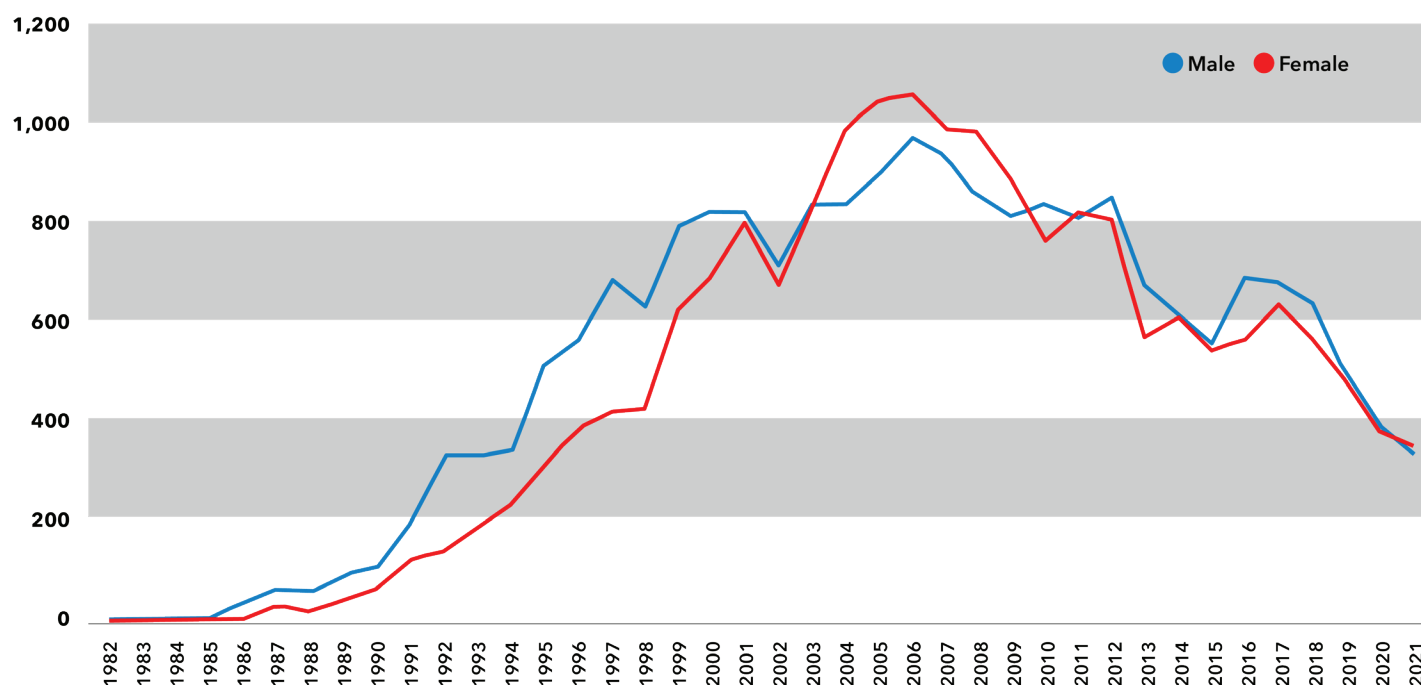


Figure 3: Cases Reported in Jamaica by Year and Sex

At the end of December 2021, almost 14% of PLHIV (4,395 persons) were unaware of their status. ART coverage has increased to 51% (percentage of persons on treatment of the diagnosed number of PLHIV) from 43% in 2017. Suppression has improved for Jamaica, moving from a low of 54% of PLHIV on ART at the end of 2017 to 78% at the end of 2021. In 2021, 749 new cases were reported to the MOHW,

a 43% decline from 2017. The ratio of the newly diagnosed based on sex at birth for male to female was approximately 1:1. Twenty-three percent (23%) of newly diagnosed females were in the 15-24 age group, while 25% of newly diagnosed males were in the 30-39 age group. St. Ann and Westmoreland had the highest case rates per 100,000 population for 2021 at 40.7 and 39.1, respectively (Figure 4).

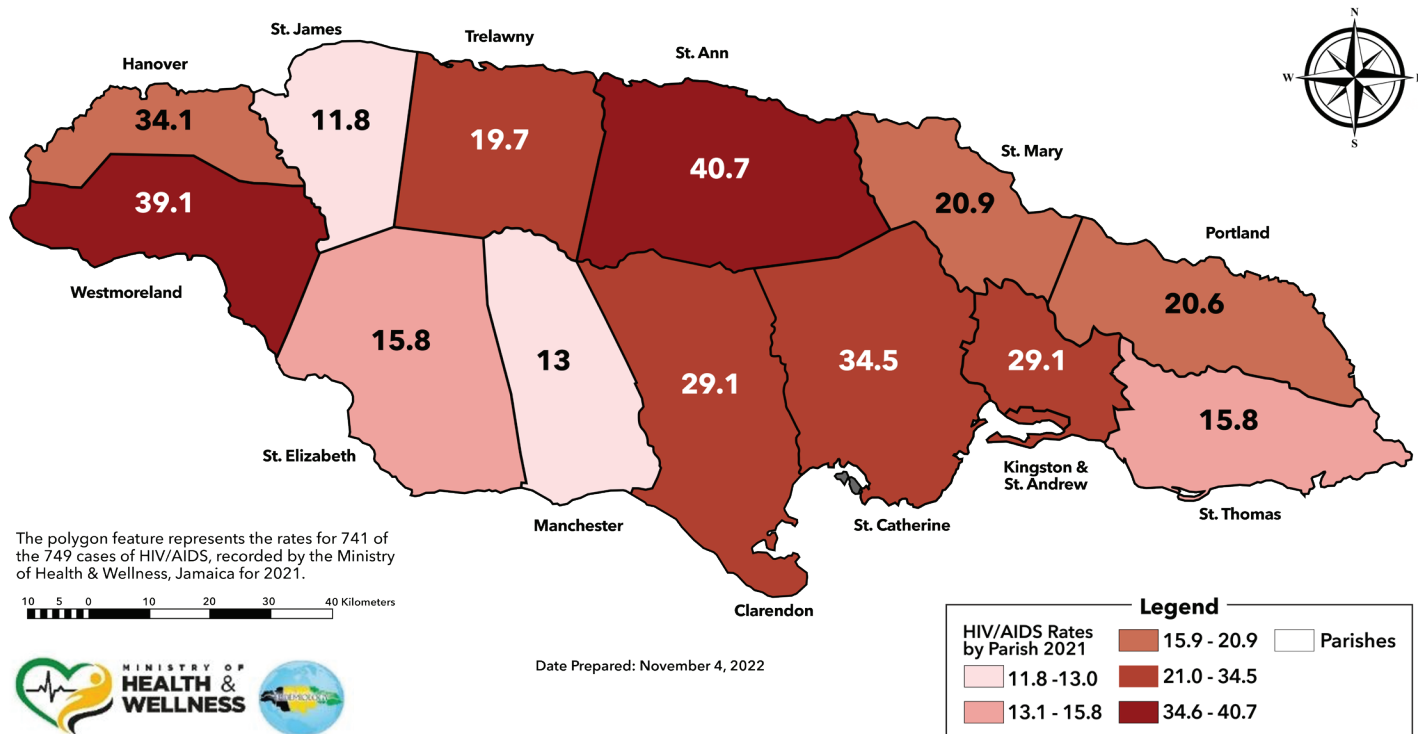


Figure 4: HIV/AIDS Rates in Jamaica by Parish, 2021

Jamaica continues to have both a generalized and a concentrated HIV epidemic. The highest proportion of new cases was identified among the general population in 2021. New HIV infections were highest among persons with a history of STIs (33%) and persons with multiple sex partners (28%).

MSM accounted for 6% of all new infections and 13% of newly diagnosed men reported sex with a sex worker. In 2021, 13% of newly diagnosed persons were adolescents and youth aged 15-24 years, which is a marked increase from 2018, when 8% of newly diagnosed persons were aged 15-24 years.

Table 2: HIV prevalence of various sub-populations in Jamaica, 1988 - 2018

	1990*	2000*	2010*	2017*
STI clinic attendees	2.5%	7.1%	Male 4.6% Female 2%	Male 7.3% Female 3%
ANC attendees	1%	2%	1%	0.9%
MSM	9.6% (Murphy et al	32.3% (Rossi 1995).	31.4% (Figueroa et al 2017	29.6% (876 study, 2018)
Transgender women			52.9%	51.0% (876 study, 2018)
Inmates		3.3% (Andrinopoulos et al. 2010),	2.5%	6.9% (Bailey, 2018)
Homeless			8.8%	13.8% (Skyers et. al, 2018) (Males 11.6%, females 26.7%)
Female sex workers	12% (Hope Enterprises 1993),	9% (Hope Enterprises 2005),	5% (Duncan 2014	2.0% (Bailey, 2017)

Table 2 shows an overall increase in HIV prevalence within most vulnerable populations, contrasting sharply with the progress made in reducing HIV prevalence among female sex workers.

The clinical cascade (Figure 5) highlights the need for continued commitment to the adolescent population.

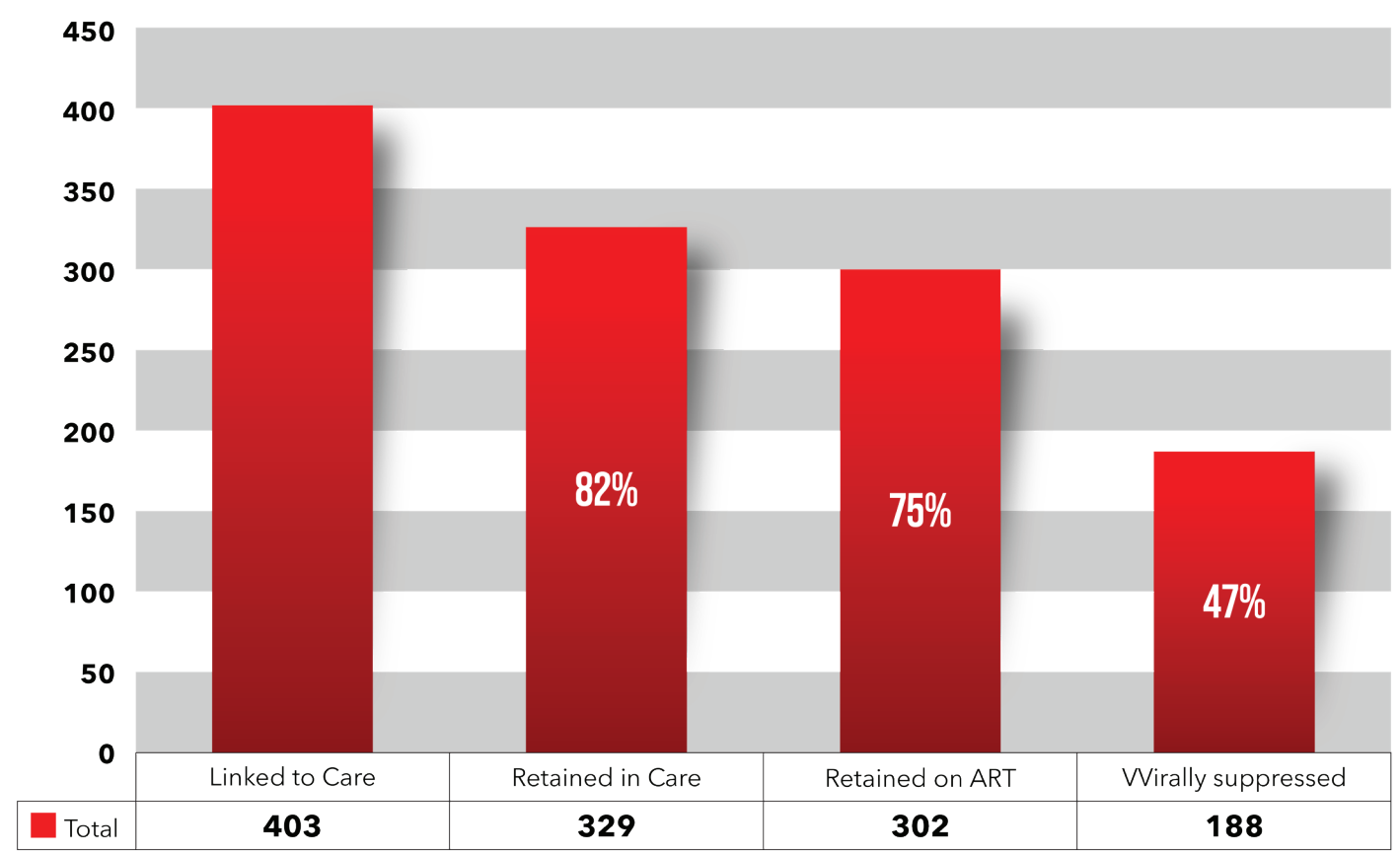


Figure 5: National Treatment Cascade: Adolescents, September 2021

There is a need for access to timely treatment, adolescent-friendly treatment sites and adherence support. The MOHW in 2016 renewed its commitment to adolescents and youth and included this cohort as a priority group in the HIV response. While the HIV prevalence among adolescents (10-19) falls well below that of the adult population at 0.5%, some groups are disproportionately affected by the epidemic (MOHW NSU data). Based on the Integrated Bio-Behavioural Surveys conducted between 2017 & 2018, the following was indicated:

- HIV prevalence among adolescent transgender individuals aged 15-19 was 27%
- HIV prevalence among young men who have sex with men (aged 15-19) was 14%
- HIV prevalence among young females (aged 15-24) who sell sex was 2.9%

SOCIAL AND CULTURAL CONTEXT

Early Sexual Initiation

Sex begins at an early age; the median age of first sex for boys has decreased from 16.5 years in 1996 to 15 years in 2017, and for girls from 18.2 years in 1996 to 17 years in 2017 (KAPB, 2017). As many as 42% of boys and 12% of girls aged 15 - 24 years reported that they initiated sex before they were 15 years of age (KAPB, 2017). The percentage of respondents who reported being the aggressor/forcing their partner into having sex increased, especially amongst the female cohort (7.5% in 2017 vs. 3.6% in 2012). Teenage pregnancy remains common in Jamaica, and data on induced abortion is limited, but it does occur among some women and adolescent girls (abortion is illegal in Jamaica).

Access to SRH services is limited for adolescents

Jamaica's Revised National Youth Policy 2017 – 2030 and the United Nations Convention on the Rights of the Child (1989) affirm the rights of youth, including minors (those aged under 16 years of age) to sexual reproductive health services. However, there is dissonance between the Law Reform (Age of Majority) Act, Offences Against the Person Act, Sexual Offences Act, the Child Care and Protection Act, and the Access to Contraceptives Policy for Minors, which hinders the accessibility of SRH services and commodities by adolescents. Current practice at service delivery points is guided by the country's legislative environment, which does not allow minors to access contraceptives without parental consent or mandatory reporting. Service provision is therefore varied and most often left to the interpretation or discretion of the providers.

There is no single policy position in the country covering the sexual and reproductive health rights of adolescents. This leads to inconsistencies in how adolescents' right to information and services are treated by different agencies. Additionally, several laws and policies provide guidelines for the provision of sexual and reproductive health information and services to minors do not always agree on issues such as age of consent and sometimes act as barriers to minors' access to SRH services. The conflict within the law and the absence of a comprehensive SRH policy creates an environment of uncertainty for both providers and adolescent service users. Consequently, advocacy efforts will be bolstered by policy dialogue with the respective Ministries, mindful that some of the legislative amendments being advocated are not the portfolio responsibility of the Ministry of Health and Wellness.

Multiple Partnerships are common among the general and key populations

Having more than one sexual partner concurrently is common in Jamaica. Population-based surveys between the periods 2004 -2017 have shown that 50% or more adult men report having from two to six sexual partners in the past year (Figueroa et al 2005, National KAP surveys 2004 - 2017, Reproductive health survey 2008). Previously 12% - 15% of adult women in these surveys reported more than one sex partner in the past year; however, recent focus groups suggest that as many as 30% - 40% of female participants may have more than one sex partner in the past year with younger women more likely to do so than older women. In 2017, 48.9% of men and 13.8% of women aged 15 - 49 years reported having more than one sexual partner in the past year

(KAPB, 2017). Persons reporting being diagnosed with an STI were significantly more likely to also have reported multiple partners in the last 12 months (17.3% in 2017 vs. 14.8% in 2012), and transactional sex (18.8% in 2017 vs. 16% in 2012).

Men are sexual risk takers

Research has shown that men generally have poor health seeking behaviours and typically seek out health care only when a situation becomes critical and unbearable. Additionally, men are more willing to engage in risky sexual practices, which manifests in situations such as having unprotected sex despite multiple partnerships (KAPB, 2017).

HIV Knowledge and Risk Perception have declined

Based on the KAPB study in 2017, only 32.9% of persons 15-24 years were able to correctly identify two ways of preventing transmission while rejecting major misconceptions. Risk perception has remained virtually unchanged, with just under three-quarters (73%) of those with multiple partners perceiving they have little or no risk of HIV infection. While persons seem to be cognizant of the need to use a condom, they (7.4%) are selectively (sometimes) opening themselves to risk by claiming that their partners "are clean" so they trust them. Persons indicating they did not like using a condom significantly increased in 2017 to 20% from 14.7% in 2012. This was primarily so among the 15-19 years' cohort (34.1% compared to 19.4% and 18.7% for the 20-24- and 25-49-years age groups respectively).

Condoms are accessible, but use is low

Most persons reported having no problems accessing condoms readily; however, actual condom use tends to be low (59%). Loving or trusting their partner (51%) was the main reason people gave for not using condoms with one's main partner in Jamaica (KAPB, 2017). Condom use during casual sex, with a non-regular sex partner or during commercial sex is generally accepted as normative behaviour, though many persons do not practice it consistently. According to the 2017 KAPB Survey, females reported last time condom use increased in married/cohabiting relationships (37.7% 2017 vs 5.7% 2012) but declined in visiting/ casual relationships (62.3% 2017 vs 94.3% 2012). While for men it was the opposite, condom use increased slightly for visiting relationships (90.0% in 2017 vs. 88% in 2012) and declined for married/cohabiting relationships (9.1% in 2017 vs. 12% in 2012). Males were more likely to agree that: condoms "reduce the quality of sex so it is better not to use at all" (43.7%), condoms reduced the spontaneity and heat (38%) and were unnecessary when the partner was

trusted (38.8%). Moreover, most Jamaican males are not circumcised and, when combined with the inconsistent use of condoms, are therefore at higher risk for STIs or HIV than men who are circumcised (Figueroa JP and Jones-Cooper C., 2010).

The poor are vulnerable and social protection is inadequate

There is a significant lack of HIV sensitive social security programmes for people living with HIV or for those most vulnerable to the disease. The poor and marginalized are vulnerable in the setting of HIV. Approximately 17% of Jamaicans are living in poverty (Planning Institute of Jamaica - PIOJ, 2017). A survey of 230 inner city youth aged 16 – 19 years in 2013 found that all except four girls ever had sex (Figueroa et al 2012). Most (91%) of the boys and 50% of the girls had sex before they were 16 years of age. 91% of the boys and 72% of the girls reported two or more lifetime sex partners while 79% of the boys and 18% of the girls reported five or more lifetime sex partners. Thirty-one percent of the boys and 45% of the girls reported one or more STI.

If one is poor and unskilled, the options for getting work are limited. Many women will therefore turn to transactional sex in order to survive or do better socially. Transactional sex was reported by 31% of respondents in the 2017 KAPB, which represents a decline when compared to 39% in 2012. (KAPB, 2017). Transactional sex is likely to be more prevalent among women of lower socio-economic status, and some women turn to commercial sex. Some women from a poor background may be tricked or forced into human trafficking.

In a 2016 report by Logie et al “Towards an understanding of structural drivers of HIV/STI and protective factors among gay, bisexual and other men who have sex with men and transgender women in Jamaica”, it was found that employment and having a higher quality of social support were protective factors against HIV infection.

In 2020, the HIV/STI TB Unit (HSTU) reviewed the impact of living support on PLHIV. The report showed, “that living support, particularly the food vouchers, serves to empower clients to purchase their own items (in most cases) and they have the option to select more of a greater variety of goods and goods which are of value to them. In addition to food, they can obtain toiletries and other household items they may need. Ultimately, vouchers serve as a temporary solution in providing nutrition to needy clients until they can become independent and self-sufficient. It also assists in beneficiaries adhering to their medication, especially among those with the belief that one cannot take the medication without a complete meal.”

Despite the importance of social protection programmes for PLHIV, a 2018 needs assessment conducted by the network of HIV positive persons in Jamaica emphasised the barriers and challenges people face in accessing Social Protection Programmes in Jamaica. Their assessment found that social protection programmes, whether public or private sector, were limited and often informal, and the persons who needed them most lacked awareness of them. Complicated procedural requirements for accessing the programmes also stopped people from benefitting from them.

PMTCT (Prevention of Mother-to-Child Transmission) is on track

While Jamaica has had significant gains in the reduction of MTCT of HIV, the country has not been able to attain elimination of mother-to-child transmission of HIV and syphilis (EMTCT). For 2021, the number of HIV positive babies was four, which was less than 50 new paediatric infections per 100,000 live births and less than the 2% HIV EMTCT target.

Women living with HIV (WLHIV) are still burdened with social issues, intimate partner violence and dependence on a partner whom they may or may not have disclosed their HIV status to (based on HSTU PMTCT case reviews of 2017 & 2018). Despite the availability of treatment care and support, women living with HIV (WLHIV) are still presenting late, non-compliant with medication and engage in practices (breastfeeding) that makes the infant susceptible to transmission.

Intimate Partner Violence is not uncommon

Another serious challenge is intimate-partner violence; 14.8% of women who were married or in a common-law relationship reported intimate partner violence in 2017 compared with 9.3% in 2012 (KAPB, 2017). According to the 2016 Women’s Health Survey for Jamaica, one in four women (25.2%) has experienced physical violence by a male partner, and 7.7% has been sexually abused by their male partner. Lifetime prevalence of intimate physical and/or sexual violence was 27.8%.

HIV stigma remains strong

Discrimination associated with HIV has reduced over the last few decades, but the stigma remains strong (Stigma Index Survey 2015, Stigma in Health Facilities 2018 and KAPB 2017; JFLAG Mystery Shoppers Survey, 2019). Most persons with HIV are concerned with how they will be viewed by other persons and fear that they may be discriminated against because of their status. Many PLHIV also have a sense of shame and guilt and feel aggrieved or

depressed. They find it very difficult to disclose their HIV status even to family members or their sexual partner. Within the health sector, discrimination against persons with HIV has declined, but some health providers and auxiliary workers in health settings breach confidentiality by gossiping about HIV-positive patients (Stigma Index Survey, 2015).

Homophobia drives the HIV epidemic underground

The stigma associated with homosexuality continues to be widespread (Figueroa et al, 2017, 876 study 2018). The risk of hostility, verbal abuse, aggression and physical violence is real for MSM and transgender persons.

In Jamaica, the issue of homophobia, self-stigma and layered stigma are obstacles to HIV prevention, treatment and care for MSM. It is difficult to reach the most-at-risk sub-populations, such as MSM with high socio-economic status, young MSM, rural MSM and MSM sex workers (2013 HIV and AIDS Legal Assessment Report for Jamaica). MSM who are low literate or experience adverse life events such as violence, jail or homelessness are at significantly higher risk of HIV (Figueroa et al, 2015). The November 2016 Community Report for Jamaica AIDS Support for Life (JASL) highlighted that "Transgender women who were HIV-positive were more likely to experience structural (homelessness, unemployment, poverty, transgender stigma), social (a history of forced sex and physical violence, lower social support), and intrapersonal (consider themselves at risk for HIV, experience depression and report poorer health) factors."

Limited progress in legislative and policy reform

Progressive steps have been made to improve the SRH/HIV-related legal and policy environment in Jamaica, such as the Parliamentary review of the Sexual Offences Act, the Offence Against the Person Act, the Domestic Violence Act and the Child Care and Protection Act (2018). HIV and AIDS are considered a priority area in the fourth Medium Term Socio-Economic Policy Framework 2018-2021, which served to propel the implementation of Jamaica's Vision 2030. A National HIV/AIDS Workplace Policy was adopted by the Jamaican Parliament in 2012, which: (i) provides protection to workers living with or affected by HIV and AIDS; (ii) helps to mitigate against HIV/AIDS-related stigma and discrimination and (iii) offers guidelines for the development and implementation of HIV/AIDS workplace policies and programmes in both the public and private, formal and informal sectors (2013 United Nations Development Programme - UNDP HIV and AIDS Legal Assessment Report).

Despite these efforts, there are gaps in the existing legislative and policy environment. Consequently, while the Constitution of Jamaica and The Charter of Fundamental Rights and Freedoms (Constitutional Amendment) Act, 2011 are indicative of the Government's commitment to the fulfilment, protection and promotion of human rights, the Charter of Fundamental Rights and Freedoms does not include a prohibition on discrimination based on health status, in particular HIV status, gender identity, sexual orientation, or disability (2013 UNDP HIV and AIDS Legal Assessment Report). This gap has implications for the success of the national HIV response.

OVERVIEW OF THE NEW 2023-2030 NATIONAL STRATEGIC PLAN FOR HIV (NSP) DEVELOPMENT PROCESS

This 2023-2030 NSP is the fourth in a series of national HIV/AIDS strategic plans. The overarching priority of the 2023-2030 NSP is to prevent HIV transmission, minimize HIV-related morbidity and mortality, and promote health and development through an effective multi-sectoral response that empowers Jamaicans to fulfil their potential and live happy, healthy, productive and peaceful lives. Intensive NSP development activities commenced in July 2019 using a bottom-up consultative and interactive approach.

SWOT Analysis

In preparation for the NSP, the overall strategies and activities of the HIV programme were reviewed. The SWOT Analysis below represents a summary of the review of the programme and the resultant NSP.

Strengths	Weaknesses
Data was thoroughly interrogated, and national quantitative size estimates for key populations are available. This strongly ensures that all HIV and AIDS programming is grounded in the most recent scientific evidence possible.	The previous NISP was not fully evaluated, so it was difficult to assess the baseline for the current document
The HIV programme acknowledges that all Jamaicans are at risk for HIV, but some sub-populations are heavily impacted.	Improving viral suppression and adherence remains difficult to address.
Importance of combination prevention, including PrEP clearly highlighted	
Heavy emphasis on linkage and retention in care using a multi-sectoral approach with a focus on the role of CSOs.	

Opportunities	Threats
Significant technical and financial support from key international donors Integration of HIV in the MOHW	Reliance on external funding Novel infectious diseases/ pandemics, wars/ political instability and acts of God. Supply chain & commodity issues
Financial commitment from the Government of Jamaica continues to increase, especially with regard to supporting human resources and ART coverage	Inconsistency between the government and HIV-related priorities
Robust multi-sectoral partnerships	Demand and supply of health care workers

The key considerations that inform the development of this NSP are declining knowledge, low risk perception and prevailing myths in the general population to include:

- High-risk sexual behaviour (sex work, transactional sex, multiple partnerships, inconsistent condom use) persists
- Gender inequity
- Inter-personal and gender-based violence, which increases vulnerability and contributes to the spread of HIV
- Poor health-seeking behaviour of men limits uptake of prevention, treatment and support services
- The governance structure of the HIV response needs strengthening
- Slow progress in legislative and policy reform
- Leadership and community mobilization are limited among key and vulnerable populations, as well as the PLHIV community
- Heavy reliance on donor funding, especially among civil society. For the 2018 GOJ fiscal year, contribution by GOJ, The Global Fund and PEPFAR was 44%, 34% and 22%, respectively.
- Limited capacity around data quality, analysis and use

The **2023 - 2030 National Strategic Plan for HIV** is framed by global, regional and national development and health agendas. The **Sustainable Development Goals (SDGs)** provide a mandate for integrated and holistic development efforts, including Target 3.3 among the health goals under SDG 3, to achieve HIV epidemic control by 2030.

- **Endorsement of the “Three Ones”:** The National HIV Program and all its partners and stakeholders endorse the UNAIDS principles of the “three ones”, namely that they work within **One National HIV Plan, One Authority** and **One Monitoring and Evaluation Framework**. All partners and stakeholders support the implementation of the National HIV Plan, that there is one overall governance body to oversee implementation of the plan and one monitoring and evaluation framework used to assess progress and impact.
- **The PAHO Plan of Action for the Prevention and Control of HIV and Sexually Transmitted Infections (2016-2021)** sets out strategic interventions and actions to be implemented to strengthen and expand the prevention and control of HIV and STIs to eliminate these infections as public health threats by 2030.
- **The UNAIDS 2025 AIDS TARGETS:**
 - 95% of PLHIV know their HIV status
 - 95% of those who know their status initiate treatment
 - 95% of PLHIV on treatment are virally suppressed
 - 95% coverage of services for eliminating vertical transmission
 - 95% of women access HIV and SRH services
 - 95% use combination prevention
 - 90% of PLHIV and people at risk are linked to people-centred and context specific integrated services
 - Less than 10% of Countries have punitive laws and policies
 - Less than 10% experience stigma and discrimination
 - Less than 10% experience gender inequalities and violence

- **The Caribbean Community (CARICOM) Strategic Plan 2015-2019** that seeks to ensure economic, social, environmental and technological resilience within the region; HIV and other critical health issues are addressed under the priority area of Building Social Resilience.
- **The Caribbean Cooperation in Health (CCH IV)** advances the CARICOM objective of enhanced functional cooperation to achieve more efficient operation of common services and activities; to promote greater understanding among its peoples and the advancement of their development; and to intensify joint activities in areas such as health.
- **The Call for Action of the 3rd Latin America and Caribbean Forum “Road to ending AIDS in LAC - Towards Sustainable Regional Fast Track Targets,”** endorsed by Caribbean countries, reiterates the need for increasing public expenditure on health to sustain responses and achieve epidemic control.
- **The Caribbean Regional Strategic Framework 2019 - 2025** articulates the vision, priorities and strategies of Caribbean states and their partners concerning controlling the HIV epidemic through their membership in the Pan Caribbean Partnership Against HIV and AIDS (PANCAP).
- **Vision 2030 Jamaica** - National Development Plan embodies the plans and processes for the realization of a collective vision, encapsulated in the statement: “Jamaica, the place of choice to live, work, raise families and do business.” The Plan has four (4) National Goals. These goals are to ensure that:
 1. Jamaicans are empowered to achieve their fullest potential.
 2. The Jamaican society is secure, cohesive and just.
 3. Jamaica’s economy is prosperous.
 4. Jamaica has a healthy natural environment.

The Health Sector Plan for Vision 2030 Jamaica presents the conceptual framework that will result in improvement in the health of every resident of Jamaica, as well as the development of the Jamaican health care delivery system to that of a first world standard by the year 2030. The sectoral Vision is “Healthy lifestyles in a healthy environment producing healthy people”.

VISION, MISSION, GOAL, STRATEGIC OBJECTIVES & GUIDING PRINCIPLES

Vision

A Jamaica free of new HIV infections and AIDS, in which all people are happy, healthy, productive and safe.

Mission

To prevent HIV, HIV- related morbidity and mortality, and promote health as well as development through an effective multi-sectoral response that empowers our people to fulfil their potential and live happy, healthy, productive and peaceful lives.

Goal

To achieve epidemic control of HIV (incidence/prevalence ratio = less than or equal to 0.03)

Strategic Objectives

1. A coordinated and strengthened multi-sectoral response, which empowers persons to practice safe, healthy lifestyles free of violence.
2. A reduction in new HIV infections by 40% by 2030 through combination prevention interventions
3. By 2025, 90% of PLHIV are aware of their status, 90% of those diagnosed are retained in care and 90% of those persons are virally suppressed and 95% of PLHIV are aware of their status, 95% of those diagnosed are retained in care and 95% of those persons are virally suppressed by 2030.
4. Strengthen the multi-sectoral framework to promote respect for the rights of all persons in relation to HIV and AIDS issues in community, policy, legislation and programmes by 2030
5. To develop and maintain a robust monitoring and evaluation framework that can guide all aspects of the HIV response

Guiding Principles

1. **Bold Leadership**

Bold action to achieve ambitious prevention, treatment and human rights targets to promote health and national development.

2. **Shared responsibility**

Strengthening mutually accountable multi-sectoral partnerships involving government, communities, civil society, and the private sector, to improve effectiveness.

3. **Social Justice**

Adopting a broad range of social strategies to reduce vulnerability and barriers to access, ensuring that no one is left behind.

4. **Human Rights**

Advancing human rights standards for equitable health and development ensuring respect and dignity for all.

5. **Gender Equality**

Promoting gender equity, strengthening agency of women, men and youth; targeting meaningful involvement of men and building balanced relationships as an essential precursor to sustainable and equitable health and development.

6. **Participation of PLHIV**

Achieving meaningful participation of persons living with HIV and vulnerable persons in all aspects of the HIV response.

7. **Evidence based**

Measures to strengthen the HIV response must be evidence based.

8. **Sustainability**

HIV strategies must be integrated within the health and social services in order to sustain the response over the long time.

STRATEGIC OBJECTIVES

SO1 - A STRENGTHENED MULTI-SECTORAL RESPONSE THAT EMPOWERS PERSONS TO PRACTICE SAFE, HEALTHY LIFESTYLES, FREE OF VIOLENCE.

Programme Coordination

The Ministry of Health and Wellness is mandated by the Government of Jamaica to coordinate and lead the implementation of the national HIV response. The HIV/STI/TB Unit and the National Family Planning Board (NFPB) jointly lead the implementation of the national HIV/STI programme. The current structure impacts programme coordination and effectiveness. A governance framework that centralizes the management and coordination of all components of the NHP in the MOHW is critical. This also includes the integration of the National Family Planning Board within the MOHW.

The integration of the National Family Planning Board into the Ministry of Health and Wellness is currently underway and will become effective once the National Family Planning Act is repealed. This will see the integration of the HIV Prevention component of the NFPB into the Health Promotion and Protection Division of the MOHW. This integration process will enhance the sustainability of the HIV response as it seeks to reduce fragmentation and streamline health promotion both at the national and community levels in keeping with the Primary Health Care Reform Plan, resulting in improved efficiencies in the utilization of scarce resources.

All the staff at central level working on HIV, other STIs and Tuberculosis will be unified within the HIV/STI/TB Unit under one overall Director of the HIV/STI Programme. The Unit will provide leadership for the development of policies, standards, monitoring and evaluation, and proposing relevant legislation along with technical guidance for both internal and external stakeholders.

In keeping with the integration of the HIV Prevention and the Health Education and Promotion teams at the subnational level, the Health Promotion and Education Unit will be expanded to include additional Health Promotion Officers who will support the operationalization of the HIV Prevention Programme in this integrated framework. The HSTU will provide the HPE Unit with the relevant strategic

information and technical guidance in keeping with the epidemiological and programmatic data.

As it relates to the Enabling Environment and Human Rights Unit of the NFPB, their work will be subsumed into the planned division of the Ministry, which should give focus to the social determinants of health. Additionally, the MOHW will re-establish the multi-sectoral HIV/AIDS Coordinating body - the National AIDS Committee (NAC). The NAC will coordinate the multi-sectoral HIV response and matters relating to the enabling environment and human rights (EEHR). Until the NAC has been re-established, however, the MOHW will continue to lead on these coordinating roles.

Service Delivery

Delivery of health services has been decentralized to the four Regional Health Authorities (RHAs). The implementation of HIV prevention and treatment is largely carried out by the RHAs. Integration will guide the approach to delivering HIV services at all levels and for all components of the response. It is critical that the staff in the field are given the leadership, tools and support to implement the plan successfully.

HIV prevention service delivery in the public health system is siloed and delivered through a network of behaviour change communication (BCC) staff dedicated to the HIV Programme. The integration of the BCC teams into the MOHW and RHA health promotion and education (HPE) structures will see a holistic health promotion response and an expansion of HIV prevention service delivery in the various settings through an integrated team. The national HPE team will be expanded to include additional BCC staff to support the expanded HPE functions at the RHA level; technical support specific to HIV will be provided to the HPE Unit by the HSTU. Regional and parish teams will deliver integrated health promotion services across the spectrum of the MOHW's health promotion priorities, inclusive of HIV prevention. Posts have been created by the Ministry of Finance and the Public Service.

HIV treatment services are currently delivered through 39 health facilities. In this strategic period, HIV treatment services will be rolled out to all health centres offering curative services, with PLHIV being able to access care akin to other chronic disease patients. Public-private partnerships will be expanded to include HIV Care access points.

Access to ARV medication is provided through both the National Health Fund's network of DrugServ pharmacies and through the twenty-one (21) private pharmacies across the island that distribute ARVs on behalf of the HSTU, MOHW. These activities are governed by a Memorandum of Understanding between the entities and the HSTU, MOHW. However, the procurement of ARVs and other HIV testing and monitoring supplies is done by the HSTU. The full integration of the procurement of ARVs and these supplies into the procurement systems of the NHF is to be effected within the first half of the 2023-30 strategic plan period.

The National Public Health Laboratory (NPHL) currently provides services for clinical and public health. It serves as a clinical laboratory for Kingston Public Hospital and Victoria Jubilee Hospital, as well as a National Reference Lab and referral laboratory in disease surveillance, vector control, food safety, and environmental health. The NPHL services include haematology, clinical chemistry, histopathology, cytopathology, microbiology, immunology, environmental health, and vector research. The national public health laboratory system encompasses the national, regional, hospital, and health centre laboratories. Development and strengthening of the tiered national public health laboratory system to support the HIV response will require the following actions:

- Strengthen HIV Drug Resistance Testing (HIVDR)
- Strengthen HIV DNA PCR for Early Infant Diagnosis (EID)
- Strengthen HIV Viral Load Testing
- Strengthen TB testing and surveillance
- Strengthen Laboratory Administrative Capacity
- Strengthen HIV Diagnosis, Treatment & Care through the introduction of HIV Recency Testing.

Currently, HIV testing is fully decentralised with the NPHL playing a quality assurance role. CD4 testing is decentralised in all RHAs except SERHA, and viral load and HIV drug resistance testing are centralised. The MOHW's plans to decentralize

operational laboratory services to the RHAs will include HIV laboratory services, and this is expected to reduce turnaround time for HIV laboratory results. Decentralization should begin during this strategic period.

Laws and Policies

There are a number of laws that need to be revised to support the multi-sector partners to achieve the strategic goals and objectives (J. Chan in the HIV and AIDS Legal Assessment Report). These include the Law Reform (Age of Majority) Act, Offences Against the Person Act and the Sexual Offences Act. The assessment makes the case for a comprehensive and enforceable anti-discrimination law that would ensure the legal protection of PLHIV in all aspects of life, including employment, education, housing and social support, among other areas. Furthermore, there have been protracted delays in the enactment of legislations to protect the confidentiality of information, including health information and the protections afforded in the HIV Regulations that accompany the long-delayed Occupational Safety and Health Act.

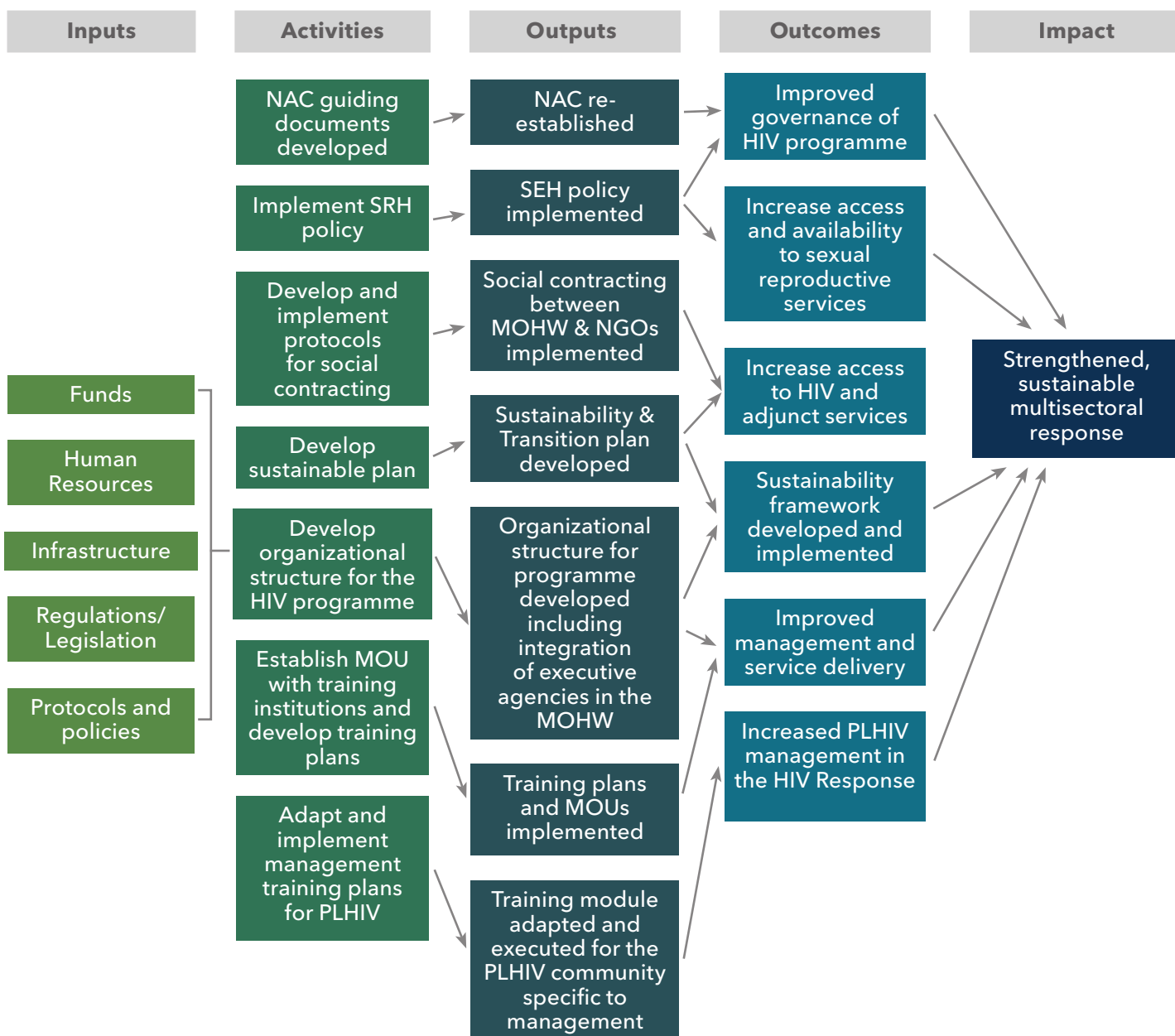
The Multi-Sectoral Response

Leadership among some CSOs including PLHIV organizations is weak, and there is an overdependence on donor funding. Efforts to strengthen the CSO towards sustainable leadership and financing for programme implementation should be a key focus.

The Government is introducing the concept of social contracting to engage CSOs to provide HIV services to key and vulnerable populations. The role of private practitioners and CSOs in reaching a subset of PLHIV and providing treatment care and support is recognized. The partnership between the national HIV programme and both private practitioners and CSOs allows the standardization of care for PLHIV through the implementation of approved policies and procedures, whilst simultaneously providing access to HIV services. The treatment database TSIS 2.0 reflects 300 persons accessing care through private physicians and 935 persons accessing care through JASL.

Logic model for Governance

Problem Statement: Fragmentation of the National HIV Response hinders efficient programme implementation



Strategies

- **Develop a governance framework that centralizes the management and coordination of all components of the NHP in the MOHW**
 - Establish the HIV/AIDS governance framework that centralizes the management and coordination of all components of the NHP in the MOHW
- **Improve legislation around sexual & reproductive health**
 - Address legislative barriers to address the sexual and reproductive health needs of the population
 - Increase advocacy for policy and legislative formulation, amendment and implementation
- **Develop human resources for programme implementation**
 - Develop human resources for programme implementation
- **Improve efficiency in the utilization of available resources**
 - Improve efficiency in the utilization of available resources
 - Identify and maintain sustainable funding sources
 - Build and strengthen partnerships for technical support
- **Increase efforts to strengthen the CSOs, religious sector and private sector towards sustainable leadership and financing for program implementation**
 - Institutionalize the role of CSOs in the national HIV response
 - Promote leadership and community mobilization among populations
 - Identify and maintain sustainable funding sources
- **Stimulate community mobilization among key and vulnerable populations including PLHIV, to foster leadership and engagement**
 - Foster leadership and engagement of the PLHIV community
 - Stimulate community mobilization among the PLHIV community
- **Integrate HIV services into the public health systems**
 - Accelerate the integration of HIV services into public health systems

Strategic Objective	Strategic Activities	Outputs
SO1: A strengthened multi-sectoral response, which empowers persons to practice safe, healthy lifestyles, free of violence.	S1.1 Develop a governance framework that centralizes the management and coordination of all components of the NHP in the MOHW.	- National AIDS Committee re-established and funded - Executive agencies absorbed within the MOHW - MOHW HIV Programme leadership re-integrated into the HIV/STI/TB Unit
	S1.2 Improve legislation around sexual & reproductive health.	SRH policy by MOHW completed & implemented
	S1.3 Develop human resources for programme implementation	- Human resources plan developed - Trained staff based on the organizational structure. - MOU developed with training institutions (to ensure alignment with the national programme needs) - Graduates in the relevant disciplines trained in HIV management as per the MOU - Training modules developed for training of staff within the in-service department of the MOHW
	S1.4 Improve efficiency in the utilization of available resources - Identify and maintain sustainable funding sources - Build and strengthen partnerships for technical support	- Continued collaboration & partnership with international agencies to provide technical and financial assistance - Procurement plans with associated risk assessments developed to mitigate possible delays in the procurement process - Sustainability and transition plan costed, adopted and implemented as Jamaica moves towards a self-sustained response

Strategic Objective	Strategic Activities	Outputs
	S1.5 Increase efforts to strengthen the CSOs, religious sector and private sector towards sustainable leadership and financing for program implementation. - Formalize the role of CSOs in the national HIV response. - Adopt the present structure utilized for the Grant management within the HSTU and utilize it for the GOJ funding of CSOs - Develop a framework to engage the CSOs for service delivery within the HIV programme - Implement agreements between the MOHW and CSOs - Monitor the services of the CSOs	Framework to guide the engagement of CSOs with MOHW & other funding agencies developed.
	S1.6 Stimulate community mobilization among key and vulnerable populations including PLHIV, to foster leadership and engagement. Establish a community led monitoring accountability mechanism.	CLM established and monitored
	S1.7 Integrate HIV services into the public health systems	- HIV prevention service delivery integrated into the Health Promotion and Education structures - HIV treatment services are fully integrated into the primary health care system - Procurement of HIV health products integrated into the National Health Fund - HIV laboratory services decentralized

Strategic Objective 1 - Framework

Strategic Activities	Time Frame								Lead Agency (ies)
	Y1	Y2	Y3	Y4	Y5	Y6	Y7	Y8	
SO1: A strengthened multi-sectoral response that empowers persons to practice safe, healthy lifestyles, free of violence.									
S1.1 Develop a governance framework that centralizes the management and coordination of all components of the NHP in the MOHW.									
Re-establish the National AIDS Committee	X	X	X						MOHW
Integration of executive agencies into the MOHW (NCDA & NFPB)	X	X	X						MOHW
Re-integration of MOHW HIV Programme leadership into the HIV/STI/TB Unit	X	X	X						MOHW
S1.2 Improve legislative formulation, amendment and implementation around sexual & reproductive health									
Draft SRH policy by MOHW developed including revision, addition and update of existing supporting legislation (this policy includes access to care for minors and key populations)	X	X	X						MOHW
Conduct legislative assessment to inform recommendations on the legislative framework needed to support the Policy's implementation.	X	X	X						MOHW
SRH policy by MOHW completed & implemented (this policy includes access to care for minors and key populations)				X	X	X	X		MOHW
Review SRH policy implementation								X	MOHW
Strengthen Technical Collaboration between MOEY & MOHW in the delivery of sexual & reproductive health	X								MOEY & MOHW

Strategic Activities	Time Frame								Lead Agency (ies)
	Y1	Y2	Y3	Y4	Y5	Y6	Y7	Y8	
S1.2 Improve legislative formulation, amendment, and implementation around sexual & reproductive health									
Formalize the mechanism by which the MOHW delivers SRH services interventions in schools		X	X	X					MOEY & MOHW
Integration of executive agencies into the MOHW (NCDA & NFPB)				X					MOHW
S 1.3 Develop human resource capacity for programme implementation									
Conduct consultancy to address the organizational structure required for the integration of the response in the PHC renewal process	X	X							MOHW
Expand the present workforce to meet the demands of the HIV response		X	X	X	X	X	X	X	
Develop an MOU with training institutions (to ensure training aligns with national programme & to have a pool for recruitment)		X	X	X					
Develop training curriculum for training institutions			X	X	X				
Training modules developed for training of staff within the in-service department of the MOHW	X	X							
Review & update training modules developed for training of staff within the in-service department of the MOHW						X			
Establish national network of trainers to deliver training based on programme needs		X	X	X					
Expand & improve the resources inclusive of infrastructure and materials to meet the demands of the response	X	X	X	X	X	X	X	X	
S 1.4 Improve efficiency in the utilization of available resources									
Establish MOUs with international agencies to provide technical and financial assistance	X	X	X	X	X	X	X	X	MOHW
Develop and implement procurement plans to be utilized by all entities within the response	X	X	X	X	X	X	X	X	MOHW, RHAs, CSOs
Revise & update the sustainability and transition plans as Jamaica moves towards a self-sustained response	X	X							MOHW
Implement sustainable funding from GOJ for key programmes within the response	X	X	X	X	X	X	X	X	MOHW

Strategic Activities	Time Frame								Lead Agency (ies)
	Y1	Y2	Y3	Y4	Y5	Y6	Y7	Y8	
S 1.5 Increase efforts to strengthen the CSOs, religious sector and private sector towards sustainable leadership and financing for program implementation									
Develop framework to engage the CSOs for service delivery within the HIV programme (social contracting, MOU, government subvention & PPP)	X								MOHW
Develop social contracting frame work to engage the CSOs for service delivery within the HIV programme	X								MOHW
Implement social contracting agreements between the MOHW and CSOs			X		X		X	X	MOHW
Monitor the services of the CSOs		X	X		X		X	X	
S1.6 Stimulate community mobilization among key and vulnerable populations including PLHIV to foster leadership and engagement.									
Adapt and implement management training plans for PLHIV	X	X	X	X	X	X	X	X	JN +
Establish community led monitoring accountability mechanism	X	X							MOHW, CSOs
Implement community led monitoring accountability mechanism			X	X	X				CSOs
Review community led monitoring accountability mechanism							X		CSOs
S1.7 Integrate HIV services into the public health systems									
Integration of HIV prevention service delivery into the Health Promotion and Education structures	X	X	X						MOHW
Full integration of HIV treatment services into the primary health care system	X	X	X						MOHW
Integration of the procurement of HIV commodities into the National Health Fund	X	X	X						MOHW NHF
Decentralization of HIV laboratory services		X	X	X	X	X			MOHW

SO2-TO REDUCE NEW HIV INFECTIONS BY 40% BY 2030 THROUGH EFFECTIVE COMBINATION PREVENTION.

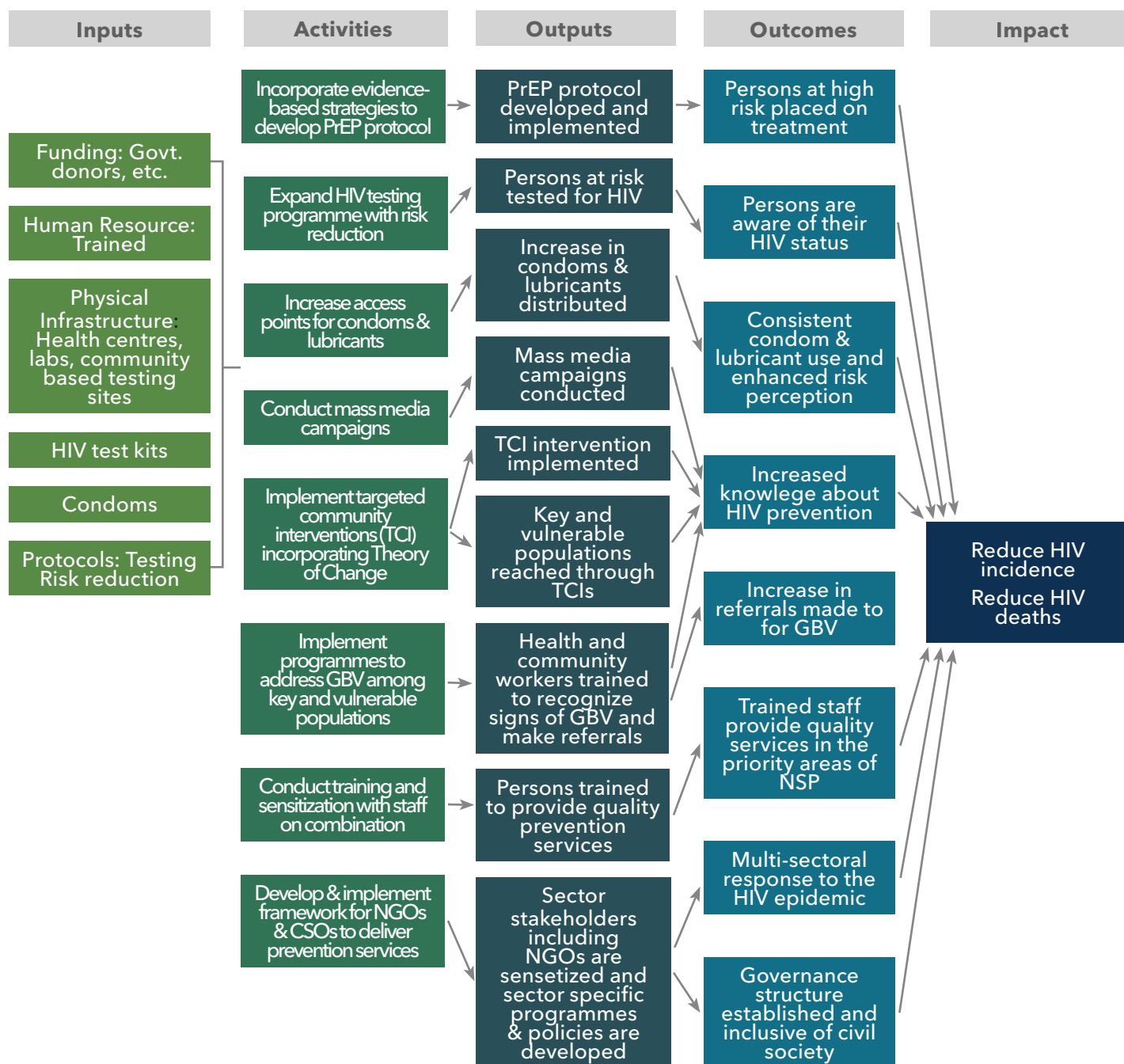
During the previous NSP, prevention efforts were focused on key populations due to their high prevalence (876 survey 2018, Bailey 2017, Skyers et. al., 2018). Various strategies were implemented to target these groups. These included site-based and venue-based interventions, empowerment and skills building workshops and one -to-one interventions. Despite strong efforts, HIV rates remained high among the key populations. In 2021, 749 new cases were identified. Highest among these were females 15-24 age group (23%) and males' 30-39 age group were at 25%. MSM accounted for 6% of all new infections and 13% of newly diagnosed men reported sex with a sex worker. While persons with a history of STIs accounted for (33%) and persons with multiple sex partners (28%) The 2017 KAPB indicated that there was a decrease in the knowledge level of respondents. Only 40.2% of respondents, 15-49 were able to provide answers to questions relating to HIV transmission.

In moving forward, targeted evidence-based combination prevention strategies and interventions are needed to achieve the long-term goal of zero new HIV infections. The combination of structural, biomedical and behavioural approaches is likely to have the greatest impact on reducing the likelihood of transmission and lessening individuals' susceptibility to acquiring new infection. The strategies to implement combination prevention include improving the package of HIV prevention services, expanding the delivery channels for these prevention services as well as improving access to sexual and reproductive health services to our adolescents and young people.

The 2017 KAPB survey assessed the level of awareness of PrEP and self-testing as well as willingness to take antiretroviral medication. Very few persons (10.4%) were aware of PrEP but 50.4% indicated a willingness to take one pill daily to reduce their chances of getting HIV. Very few persons (12.8%) had knowledge of HIV self-tests and only 2% had administered same. The majority (69.1%) of the sample however indicated a likelihood that they could consider using this method of testing in the future.

Logic model for HIV prevention

Problem Statement: High risk behaviors such as multiple partners, inconsistent condom use, along with adequate risk perception and negotiation skills continue to fuel the HIV epidemic in Jamaica



Strategies

- **Improving the package of HIV prevention services**
 - Design and implement a PrEP programme, inclusive of a promotional campaign for PrEP
 - Strengthen interventions targeting men, boys, adolescent girls and young women
 - Strengthen the quality of current programmes and design differentiated prevention programmes for key and vulnerable populations (SW, TGW, AGYW, MSM, Homeless, Inmates)
 - Training of service providers in identifying and referring incidents of GBV
 - Strengthen the competence of Programme staff to deliver prevention services
- **Expand the delivery channels for HIV Prevention Services**
 - Traditional & Social media campaigns
 - CSO and multi-sectoral partnerships
 - Peer-led (male, adolescent and youth 15-24) risk conversations
 - Online outreach via social media platforms for key and vulnerable populations
- **Expand testing services**
 - Self-testing
 - Partner referral testing
 - Targeted testing for key and vulnerable populations
 - Testing in juvenile and state facilities
 - Testing in youth settings
 - Fixed testing sites in non-clinical locations
 - Provider Initiated Testing and Counselling (PITC)
- **Condom social marketing**
 - Condom and lubricant promotion
 - Collaboration with private condom marketers to promote condom use
 - Condom media campaign
- **Improving access to SRH services for men and adolescent and youth**
 - Build the capacity of healthcare providers to provide male-centred services and adolescent-friendly services
 - Develop male specific outreach activities
 - Develop adolescent and youth-friendly services
 - Engage men and women in peer-led risk assessment conversations
 - Male and female focused media campaigns

Strategic Objective	Strategies	Outputs
SO 2: To reduce new HIV infections by 40% by 2025 through effective combination prevention.	S2.1 Improving the package of HIV prevention services to scale up activities targeting vulnerable populations S2.1a Design and implement a PrEP programme, inclusive of a promotional campaign for the PrEP programme S2.1b Strengthen interventions targeting low-risk women, men and boys and youth S2.1c Strengthen the quality of current programmes and design differentiated prevention programmes for key and vulnerable populations including the Homeless and disabled 2.1d Training of service providers in identifying and referring incidents of GBV S2.1e Strengthen the competence of staff to deliver prevention services	- Standardized package of prevention services developed - PrEP Programme Cabinet-approved - PrEP programme designed and implemented for continuous and event driven PrEP - Promotional campaign developed for PrEP reaching persons at high risk - Interventions implemented in high-risk communities with key and vulnerable populations. - Engage stakeholders in the development of prevention programmes relevant and appropriate to engage with members of the disabled population - Staff trained and able to identify signs of GBV and make the necessary referral - Conduct training of 75% of service providers in health facilities in identifying and referring incidents of GBV - Staff trained
	S2.2 Expand the delivery channels for HIV prevention services using: - Traditional & Social media campaigns - CSO and multi-sectoral partnerships - Peer-led (male, adolescent and youth 15-24) risk conversations - Online outreach via social media platforms for key and vulnerable populations	- Media campaigns developed - CSOs delivering prevention services - Peers trained to conduct risk conversations reaching peers - Peers reached out through online outreach using various social media platforms

Strategic Objective	Strategies	Outputs
	S2.3 Expand testing services S2.3a Self-testing 2.3b Partner referral testing 2.3c Targeted testing for key and vulnerable populations 2.3d Testing in Juvenile and State facilities 2.3e Testing in a youth setting 2.3f Fixed testing sites in non-clinical locations 2.3g Provider Initiated Testing and Counselling (PITC)	- Persons accessing HIV testing services - PITC offered in male clinics in hospitals (A&E) and outreach sites
	S2.4 Condom & lubricant social marketing 2.4a Condom and lubricant promotion 2.4b Collaboration with private condom marketers to promote condom use 2.4c Condom media campaigns	- Promotion of condoms and lubricants - design and place 2 condom media campaigns - Collaborate with 2 condom marketers to promote condom use and increase distribution
	S2.5 Improve access to SRH services for men, boys and low risk women S2.5a Build the capacity of Health care providers to provide male-centred services 2.5b Develop male specific outreach activities 2.5c Engage low risk women in peer-led risk assessment conversations 2.5d Male and female focused media campaigns	- Increase the number of key and vulnerable populations accessing SRH services - Health care providers trained to provide male-centred services adolescent friendly services - Male specific outreach activities implemented in communities - Offer service at male, adolescent and youth centred sites - Programs implemented targeting men and young women - Male and female media campaigns developed

Strategic Objective 2 - Framework

Strategic Activities	Time Frame								Lead Agency (ies)
	Y1	Y2	Y3	Y4	Y5	Y6	Y7	Y8	
SO 2 To reduce new HIV infections by 40% by 2030 through effective combination prevention									
SO 2.1 Improved package of services for key and vulnerable populations									
Condom quantification and procurement	X	X	X	X	X	x	x	x	MOHW NHF
Develop and implement a monitoring framework for condom distribution	X	X	X	X	X	x	x	x	MOHW
Revise the standardized package of services to reach populations	X		X			x			
Revise Standard Operating Procedures developed to reach targeted populations	X		X			x			
Develop and revise protocols to guide the implementation of programmes	X		X			x			
Revise standardized strategies developed to reach women, men, boys, and youth	X		x			x			
Conduct capacity building of the staff of RHAs and CSOs to engage the key and vulnerable population	x	X	X	X	X	x	x	x	
Conduct ongoing monitoring of the implementation of strategies	X	X	X	X	X	x	x	x	
Prepare and submit a concept note for the PrEP Programme	X								MOHW
Cabinet approval for the PrEP Programme	X	X							
Conduct training and implement PrEP strategy across CSO and RHA		X	X	X	X	X	X	X	
Plan & Budget for an additional 100 persons per year to access PrEP, with a total of 800 persons at the end of 8 years		X	X	X	X	x	x	x	
Implement and monitor the PrEP uptake of participants	X	x	x	x	x	x	x	x	
Evaluate the PrEP programme (Process and Impact evaluation)				x				x	
Strengthen the competence of staff to deliver prevention services	X	X							
SO 2.2 Expand the delivery channels for HIV prevention services using Mass & Social media CSO and multi-sectoral partnerships Peer-led risk conversations CSO and multi-sectoral partnerships Peer-led risk conversations									
Develop and place a media campaign	x			X					MOHW
Capacity building of non-traditional stakeholders to provide prevention services	x	X	X	X	x	x	x	x	MOHW/CSOs
Revise conversation guides and protocol for conducting risk assessment and risk reduction conversations	X		x			x			MOHW

Strategic Activities	Time Frame								Lead Agency (ies)
	Y1	Y2	Y3	Y4	Y5	Y6	Y7	Y8	
Establishing a cadre of peers to enable them to engage their peers in risk assessment and risk reduction conversations	X	X	X	X	X				MOHW
Revise monitoring tool to assess peers in conducting sessions	X								MOHW
Assessment of Peers in conducting risk reduction & assessment sessions		X		X					MOHW
Revise protocol and tools for the implementation and monitoring of online outreach	x	x							MOHW
2.3 Expand testing services									
Revision of outreach testing protocols	X		X						MOHW
Develop and place a public awareness campaign	X		X	X					
Targeted testing in juvenile facilities and state care	X	X	X	X	X				
Targeted testing in adolescent and youth settings									
Develop SOPs and protocols for fixed testing sites in non-clinical locations	X	X	X						MOHW
Scale up Provider Initiated Testing and counselling (PITC)	X	X	X	X	X	X	X	X	MOHW
2.4 Condom social marketing									
Develop and place a campaign using all media on condoms and lubricants		X		X					MOHW
Engage private sector condom marketing companies for condom promotion and distribution	X	X	X	X	X				
2.5 Improving access to SRH services for men, adolescent girls, and young women									
Provide SRH services to key and vulnerable populations	X	X	X	X	X	X	X	X	MOHW
HCW trained to provide male and adolescent-friendly services	X	X	X	X	X	X	X	X	
• Male centred and adolescent-friendly SRH activities implemented in island-wide targeting male, adolescent, and youth-centred sites (places of entertainment and work)	X	X	X	X	X	X	X	X	
Collaborate with SDC and other stakeholders to develop and implement programmes to reach OSY		X	X	X	X	X	X	X	MOHW
Strengthen partnerships with FBOs, Professional and civic organizations and workplaces to implement peer-led interventions for adolescent girls and young women	X	X	X	X	X	X	X	X	MOHW
Develop and place male and female focused media campaigns		X		X					MOHW

**SO3- BY 2025,
90% OF PLHIV ARE AWARE OF THEIR STATUS,
90% OF THOSE DIAGNOSED ARE RETAINED ON ART AND
90% OF THOSE PERSONS ARE VIRALLY SUPPRESSED AND
BY 2030,
95% OF PLHIV ARE AWARE OF THEIR STATUS,
95% OF THOSE DIAGNOSED ARE RETAINED ON ART AND
95% OF THOSE PERSONS ARE VIRALLY SUPPRESSED.**

Jamaica has improved in all pillars of the cascade at a steady pace. The most recent data from the Ministry of Health & Wellness reflects that there were 32,586 PLHIV in Jamaica as of December 2021, 27,605 (86%) that have been diagnosed, 14,030 (51%) of those diagnosed have been retained on

ART (44% of all PLHIV) and 10,904 (78%) of those on antiretroviral treatment were virally suppressed (Table 3). However, approximately 4,981 PLHIV were unaware of their status, 3,991 have defaulted from care, and 3,126 on ART are unsuppressed. (MOHW NSU and HSTU data).

Table 3: Projected estimates of total PLHIV and the number to be put on ART annually

Year	No. of PLHIV	New on ART	Total PLHIV on ART
2020	32,609	1551	13,000
2021	32,586	4,000	14,030
2022	30,128	3,100	16,800
2023	30,507	3,100	19,600
2024	30,914	3,000	22,400
2025	31,287	3,000	25,400
2026	31,427	900	26,300
2027	31,228	700	27,000
2028	30,864	550	27,550
2029	30,304	410	27,960
2030	29,662	320	28,280

Source: Spectrum Estimates 2022

It is projected that there will be approximately 31,000 PLHIV in Jamaica by 2025. The assumptions underlying these projections are that the number of new HIV infections will decline from 1,500 in 2020 to 400 in 2030; the number of deaths will decline from 800 in 2021 to 100 in 2030, and that an average of 5% of PLHIV will migrate from Jamaica each year. To meet the 90-90-90 targets by 2025, we will need to diagnose 28,158 PLHIV, place 25,342 of them on antiretroviral medication and ensure that 22,808 PLHIV are virally suppressed. In order to achieve the 2nd 90, an average of 3,000 PLHIV must be put on ART each year between 2022 and 30.

The above projections are made on the basis of the UNAIDS estimates derived from the mathematical computer modelling of various indicators captured by the HIV case-based surveillance system and

monitoring and evaluation mechanisms. These include, but are not limited to, new HIV diagnosis, HIV/AIDS related deaths, HIV exposed infants who become positive, and other indicators that are tracked on the HIV continuum of care treatment cascade.

During 2021, approximately 154,067 HIV tests were done in Jamaica, resulting in 2,528 positive tests and 791 newly diagnosed PLHIV. To identify 5,676 persons living with HIV who are unaware of their status, as many as 500,000 adults (35% of persons 15 - 49 years) will need to be tested. The majority of these tests must be targeted at persons who are more at risk. Testing yields are highest among contacts of PLHIV, STI Clinic attendees and hospital admissions (Table 4)

Table 4: Positivity Yield of different testing modalities

Testing Modality	No. HIV tests done	No +ve	% +ve
ANC Testing	31685	103	0.33%
PITC	36703	700	1.9%
Index Testing	541	234	43%
Outreach Testing	44854	354	0.79%

HIV testing will be targeted towards men and male dominated spaces; testing of contacts of PLHIV will be scaled up in addition to ensuring that testing in STI clinics and hospital admissions are brought to scale.

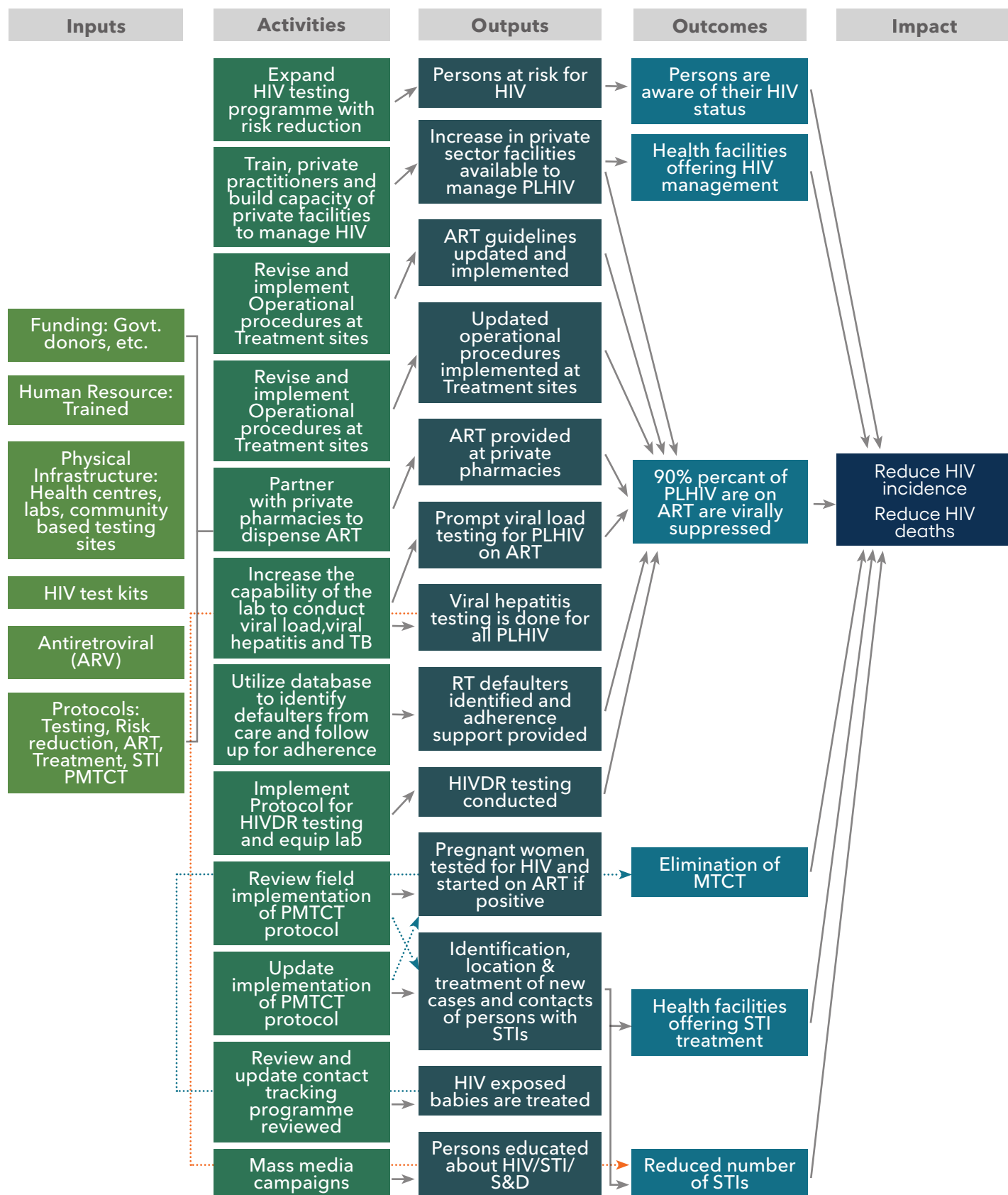
To reach PLHIV never linked to care, an entry to care activity was utilised for the persons newly diagnosed between 2015 - 2019 to engage these persons. Of the 6060 persons newly diagnosed during that period, 16% were never linked to care, of which the majority of these persons were either confirmed dead or had migrated. This entry to care campaign will continue to engage these persons never been linked to care in 5-year specific age bands. Increasing access points for care in the public and private settings, as well as community-based organizations are means of addressing linkage and retention in care. Ensuring that patients remain in care through engagement of staff and communities in HIV literacy, stigma & discrimination and enabling PLHIV to navigate the health care system remain the key means of creating a suitable environment for PLHIV to receive care. Differentiated care - tailoring care for PLHIV based on their needs will be applied across the board, ranging from intense adherence and psychosocial programmes for difficult to manage PLHIV, to longer appointments and multi-month ARV dispensing for the stable patient. Targeted strategies for retention and adherence for the paediatric population, adolescents and FSWs will be designed through operational research and differentiated models of care.

Elimination of mother-to-child transmission has not been achieved by Jamaica. Challenges identified in the PMTCT programme include gaps in service delivery, stigma and discrimination, as well as issues surrounding disclosure of HIV status and socioeconomic barriers. These issues will be addressed through customizing the PMTCT programme to meet the specialized needs of each pregnant woman through differentiated models of care while addressing the SRH needs of PLHIV women of reproductive age.

Viral load testing is challenged due to non-adherence to testing protocols as well as laboratory-based challenges. Confirmed viral suppression is also lowest in the paediatric and adolescent age groups, along with PLHIV 20 - 24 years old; within the 0 - 24 age group, only 1 in 3 PLHIV have confirmed viral suppression. Viral suppression rates were noted to increase with age (2020 HIV annual report). Other key and vulnerable groups with low viral suppression include female sex workers, prisoners and the disabled. In addition to adherence strategies with a focus on young people and the aforementioned key and vulnerable groups, efforts to increase compliance with testing protocols and reduce disruptions in laboratory testing will be used. Optimised ARV regimens and HIV Drug Resistance testing will also be employed to improve viral suppression.

Logic model for Treatment

Problem Statement: Access to antiretroviral treatment and viral load suppression is suboptimal among PLHIV



Strategies

- **Improving testing services**

- PITC
- Partner testing
- Targeted testing for men and other groups
- Targeted testing in STI clinics, A&E departments & Adult hospital admissions

- **Retention on ART**

- Improve linkage to care
 - Immediate linkage to care for the newly diagnosed
 - Entry to care campaign for those diagnosed but never linked to care
 - Increase ART access points through public & private avenues
 - Improve treatment literacy
- Retention in care
 - Return to Care activities for LTFU PLHIV
 - Differentiated model of care for PLHIV in care
 - Paediatric population
 - Adolescents
 - Pregnant PLHIV
 - FSW
 - Men
- Targeted psychological and social support for PLHIV with the greatest need; utilizing U=You=U and disclosure

- **Improve viral suppression**

- Continue implementation of adherence strategies, including peer, social, psychological and SMS reminders
- Ensure uninterrupted access of ARVs
- Annual review of ARV regimens
- Decentralization of viral load testing
- Scale up drug resistance testing
- Scale up high viral load clinics

Strategic Objective	Strategic Activities	Outputs
SO 3: By 2025, 90% of PLHIV are aware of their status, 90% of those diagnosed are retained on ART and 90% of those persons are virally suppressed and by 2030, 95% of PLHIV are aware of their status, 95% of those diagnosed are retained in care and 95% of those persons are virally suppressed	S3.1 Improving diagnosis using: • PITC • Partner testing • Targeted testing for men and other groups • Targeted testing in STI clinics, A&E departments & Adult hospital admissions	• Partner testing - Increase the cadre of CIs to facilitate contact tracing (2 cohorts of 25 trained in the 5 yr. period) • 75% of partners identified as tested • 100% of STI clients tested • 80% of hospital admissions were tested
	S3.2 Improve retention on ART by: S3.2a Improve linkage to care • Decreasing time for patients to be linked to care • Immediate linkage to care for the newly diagnosed • Entry to care campaign for those diagnosed but never linked to care, • Increase ART access points through public & private avenues S3.2b Improve retention in care • Improve treatment literacy • Continue return to care activities for persons LTFU and defaulted • Implement a differentiated model of care for PLHIV in care, paediatric, adolescents, pregnant PLHIV, the aging population with comorbidities and FSWs. • Targeted psychological and social support for PLHIV with the greatest need; utilizing U=You=U and disclosure	• 95% of persons on ART by 2030 • Increase in the number of CIs by 25% in 7 years. • Increase in treatment access points - all Type 3 - 5 HC can offer basic PLHIV management • 50% of persons never linked to care were reached during the entry to care campaign • All existing treatment sites can offer a minimum of 3 days of evening clinic • Intense adherence Clinics implemented at all treatment sites • Longer appointments and multi-month scripts for stable patients - utilize the rapid care pathway model at treatment sites • IEC material created & developed • 20% of persons lost to care were retained in care

Strategic Objective	Strategic Activities	Outputs
	S3.3 Improve viral suppression - Continue implementation of adherence strategies, including peer, social, psychological and SMS reminders - Ensure uninterrupted access of ARTs - Annual review of ART regimens - Decentralization of viral load testing - Scale up drug resistance testing - Scale up high viral load clinics	- Formalizing Living Support & provision of a mechanism for the delivery of same - Mental Health First Aid manual in place in year 1. Training conducted for 80% of the treatment site staff. 95% of PLHIV virally suppressed. - Decentralization of VL testing. Additional VL machines for the Comprehensive Health Centre, Slipe Pen Rd. & CRH - Implement HIVDR surveillance plan - Increase accessibility, as stated previously

Strategic Objective 3 - Framework

Strategic Activities	Time Frame								Lead Agency (ies)
	Y1	Y2	Y3	Y4	Y5	Y6	Y7	Y8	
SO 3: By 2025, 90% of PLHIV are aware of their status, 90% of those diagnosed are retained in care and 90% of those persons are virally suppressed and by 2030, 95% of PLHIV are aware of their status, 95% of those diagnosed are retained in care and 95% of those persons are virally suppressed by 2030									
3.1 Improve diagnosis using different modalities to attain the first 90									
Forecast, quantify and procure test kits and sundries for HIV testing in health care settings and outreach	X	X	X	X	X	X	X	X	
Formalize the CI curriculum and adopt it in the in-service department of MOHW	X	X							
Train three cohorts of Contact Investigators	X			X			X		
3.2 Improve retention on ART									
Entry to Care activities for persons diagnosed who have never been linked	X	X	X	X					MOHW
Increase access points for care by integrating HIV care in all HCs offering curative services, and private care	X	X	X	X	X				
Create male specific clinic spaces for male PLHIV in each Parish		X	X	X	X				
Expand adolescent-friendly spaces for ALHIV in each parish		X	X	X	X				
Increase the number of: • Contact Investigators									

Strategic Activities	Time Frame								Lead Agency (ies)
	Y1	Y2	Y3	Y4	Y5	Y6	Y7	Y8	
Increase the cadre of: - Psychologist inclusive of Paeds & adolescents - Case Managers - Social Workers - Adherence Counsellors - Community Facilitators									MOHW
Develop protocols to guide the delivery of service for the psychosocial team	X					X			
Forecast, quantify and procure ARVs for existing and new patients based on the projection of 10,000 additional persons on ARV at the end of the period	X	X	X	X	X	X	X	X	
Increase access points to care	X	X	X						
Implement evening clinics at all treatment sites for a minimum of three days per week	X	X	X	X	X	X	X	X	
3.3 Improve Viral Suppression									
Improve the capacity of NPHL to offer VL testing		X							MOHW NHF
Forecast, quantify and procure testing modalities	X	X	X	X	X	X	X	X	
Decentralise VL testing to WRHA & SERHA			X	X					
Enhanced Adherence Clinics implemented at all treatment sites			X						
Longer appointments and multi-month scripts for stable patients - utilise the rapid care pathway model at treatment sites	X								
Forecast, quantify and procure reagents for routine HIVDR testing	X	X	X	X	X	X	X	X	
Implement a Surveillance plan for DR	X	X	X	X	X	X	X	X	

SO4 - STRENGTHEN THE MULTI-SECTORAL FRAMEWORK TO PROMOTE RESPECT FOR THE RIGHTS OF ALL PERSONS IN RELATION TO HIV AND AIDS ISSUES IN COMMUNITY, POLICY, LEGISLATION AND PROGRAMMES BY 2030

The promotion, protection and fulfilment of rights and fundamental freedoms for all, including persons living with HIV and AIDS and other key and vulnerable populations, is essential to reduce vulnerability to HIV and AIDS. The achievement of optimal health is dependent upon existing provisions for the underlying determinants of health, which are encapsulated in other human rights, such as the right to food, housing, work, education, information, and participation, among others. An enabling environment framework, freedom from discrimination and the support of various biomedical, social, psychosocial and economic approaches are essential and central components of an effective HIV response.

The 2023-30 NSP reflects efforts by the Ministry of Health and Wellness through the National Family Planning Board, and its multi-sector partners in the public and private sectors, as well as civil society, including faith-based organizations. Under the Enabling Environment Component, the respective collaborations/partnerships aim to overcome the major rights-related barriers for all persons in Jamaica to access services that support HIV prevention, treatment, care and support. The key strategy to lead this component will be the reestablishment of the National AIDS Committee, which will be tasked with the coordination of the multi-sectoral response as well as undertaking policy advocacy to improve access to services for all. The integration of the work of the Enabling Environment component within the Ministry's policy division will support the prioritization of required policy changes, thereby reducing the wait time for policy reform matters to be placed on the Ministry's policy agenda. In addition, it will promote increased interface with the Standards and Regulations unit and the Complaints desk of the MoHW that monitors quality of care and initiates redress as the Ministry moves to become more patient-focused.

Overall, the strategy recognizes that discrimination against people living with HIV and AIDS and key populations violates their rights and is counterproductive to an effective and enabling response. Significantly, the strategy moves to

operationalize the human rights principle that all Jamaicans have a responsibility to respect the rights of others and affirm the dignity of all persons, including persons living with HIV, as well as key and vulnerable populations.

A community systems strengthening approach is reflected as capacity building is to be pursued to develop informed, capable and coordinated communities, and community-based organizations, groups, networks and structures to improve health outcomes. An element of a rights-based approach is meaningful participation. This is reflected in the initiatives to empower and support the meaningful participation of people living with and affected by HIV and AIDS, as well as key and vulnerable populations in the design, delivery, monitoring and evaluation of policies, services and activities related to prevention, treatment, care and support. Additionally, this approach will enhance activities to increase accountability to a rights-based approach to service delivery and improve access to justice for persons living with HIV and key and vulnerable populations.

Jamaica has made progress in developing or amending laws, policies, guidelines and protocols to create an enabling environment for the HIV response; however, barriers to the successful implementation of these policies and guidelines remain. There has been limited success in the creation of an enabling policy and legal environment to support the efforts being made to eliminate new HIV infections (Global Fund, Baseline Assessment - Jamaica 2019).

Progressive steps have been made to improve the SRH/HIV-related legal and policy environment in Jamaica, such as:

- The Government of Jamaica has demonstrated its commitment to a rights-based approach by ratifying seven of the nine core human rights instruments, including the International Covenant on Economic, Social and Cultural Rights, the International Covenant on Civil and Political Rights and the Convention on the Rights of Persons with Disabilities.
- The Charter of Fundamental Rights and Freedoms (Constitutional Amendment) Act, 2011
 - amended the Constitution of Jamaica and incorporated the principles of the Universal Declaration of Human Rights (UDHR). Of note, Section 13(2) (b) states that "Parliament shall pass no law and no organ of the State shall take any action which abrogates, abridges or infringes those rights."

- As a signatory to core human rights instruments, Jamaica is required to report on its human rights obligations. Consequently, the government has employed an inter-ministerial approach to the preparation of reports and responses related to the fulfilment of its rights obligations and has taken steps to establish an Inter-Ministerial Committee on Human Rights to improve the coordination and standardization of its reporting.
- Review of the Sexual Offences and related Acts: the Offence Against the Person Act, the Domestic Violence Act and the Child Care and Protection Act- performed by a Joint Select Committee of Parliament and the submission of notable recommendations to Parliament aimed at strengthening the laws and increasing access to justice.
- Prioritization of HIV and AIDS as reflected in the second Medium Term Socio-Economic Policy Framework 2012-2015, which served to propel the implementation of Jamaica's Vision 2030.
- Approval of a National HIV/AIDS Workplace Policy by the Jamaican Parliament in 2012, which: (i) provides protection to workers living with or affected by HIV and AIDS; (ii) helps to mitigate against HIV/AIDS-related stigma and discrimination and (iii) offers guidelines for the development and implementation of HIV/AIDS workplace policies and programmes in both the public and private, formal and informal sectors (2013 UNDP HIV and AIDS Legal Assessment Report).
- The completion by the Ministry of Justice (MOJ) of a Cabinet Submission in 2018 to establish a National Human Rights Institute (NHRI) based on the Paris Principles to advance the rights of Jamaicans by monitoring and ensuring Jamaica's compliance with its human rights obligations.

Despite these efforts, there are gaps/challenges in the existing legislative and policy environment. The findings of a legislative assessment conducted in 2013 highlighted that Jamaica has outdated HIV-related national policies that have limited enforcement and concomitant accountability.

While the Constitution of Jamaica and The Charter of Fundamental Rights and Freedoms (Constitutional Amendment) Act, 2011 are indicative of the Government's commitment to the fulfilment, protection and promotion of human rights, it does not include a prohibition on discrimination based on health status, in particular HIV status, gender identity, sexual orientation, or disability (2013 UNDP HIV and AIDS Legal Assessment Report). This gap has implications for the success of the national HIV response in Jamaica.

More recently, the findings of the 2019 Global Fund Human Rights Baseline Assessment Report highlighted that there are stigmatizing attitudes and discriminatory practices across various settings and communities in Jamaica, including justice, education, faith-based community and workplaces. This signals the need for interventions to reduce stigma and discrimination in all settings.

While stigma and discrimination associated with HIV have been reduced to less than what it was a few decades ago, S&D remains prevalent (See: Stigma Index Survey 2015; Stigma in Health Facilities 2018 and KAPB 2017; MSM Internalized Stigma Study; JFLAG Mystery Shopper's Survey). The stigma associated with homosexuality remains widespread (Figueroa et al, 2017, 876 study 2018). Within the health sector, discrimination against persons living with HIV has declined; however, continued human rights education is critical, based on the continued reports of rights violations by some health care staff, medical and non-medical, such as the right to privacy and equitable and humane treatment (Stigma Index Survey 2015). Similarly, human rights interventions need to be pursued in all settings, as there is a general paucity in awareness of rights among the citizenry, with some among the population who equate discussions of human rights with "gay rights."

A comprehensive approach to the achievement of optimal sexual and reproductive health also requires addressing the factors that drive adverse sexual and reproductive health, among which are poverty, social marginalization, gender norms, equity, gender inequality and sexual and physical abuse.

Logic model for Enabling Environment & Rights

Problem Statement: Stigma and discrimination are major barriers to accessing HIV prevention, treatment, care and support services



Strategies

- **Reduce stigma & discrimination**

- Promote the need for legislative reform to address issues affecting epidemic control
- Review and update outdated HIV-related national policies
- Institutionalize modules on HIV, rights-based approaches, stigma and discrimination and gender into pre- and in-service curricula, orientation programme for duty-bearers, service providers and professional associations
- Train service providers in rights-based and people-centred approaches for the provision and delivery of services
- Implement traditional and social media campaigns to raise awareness of the harmful consequences of stigma and discrimination
- Scale-up efforts to build the capacity of PLHIV to be involved in the response and improve their healthcare
- Scale up of stigma and discrimination reduction interventions within the community, faith-based, workplace, education, health and justice settings.
- Conduct routine knowledge, attitudes, practices and behaviour surveys to assess Stigma and discrimination and the gaps in the legal environment

- **Community-Led Monitoring**

- Strengthen the partnership between service providers and service users in service improvement efforts in education, health, justice, workplaces and communities

- **Increase access to justice**

- Sensitize key stakeholders including law enforcement agents on PLHIV and key and vulnerable populations related issues in relation to reporting, investigations and redress processes
- Strengthen the existing reporting and redress mechanisms to increase the number of cases/complaints for which redress is effected
- Promote legal literacy and increase access to HIV-related legal services, such as legal information and referrals
- Engage community leaders to improve awareness of people-centred approaches and quality standards in accessing services
- Increase awareness of alternative forms of dispute resolution to effect redress for HIV-related discrimination
- Promote the implementation of the Health in All Policies approach
- Train relevant stakeholders to navigate reporting and redress mechanisms

- **Gender-Based Violence (GBV) and Inequalities reduced**

- Integrate modules on GBV and inequality in training/sensitization sessions
- Promote multi-sectoral partnerships/collaboration for the implementation of GBV interventions
- Implement programmes to increase the educational and economic empowerment of women living with or vulnerable to HIV infection
- Build the capacity of policy makers, law enforcement agents and other key stakeholders to address gender-based violence

- **Strengthen Community Systems**

- Strengthen networks, linkages, coalitions and partnerships with communities to avoid duplication of interventions
- Foster full partnership between community actors and national systems especially in the health, education, justice and social sectors

- **Address barriers to vulnerable and key populations' access to social protection services**

- Improve access to social protection services by PLHIV, key and vulnerable populations
- Promote social protection policies and programmes that are concordant and HIV sensitive
- Promote sustained funding of social protection programmes through the use of domestic resources
- Promote the incorporation of the needs of people living with HIV into the disaster risk management plans

Strategic Objective	Strategic Activities	Outputs
SO 4: To promote respect for the rights of all persons in relation to HIV and AIDS issues in community, policy, legislation and programmes	S 4.1 Reduce stigma & discrimination S 4.1a. Mass media campaigns and advocacy initiatives for S&D reduction S 4.1b. Sensitize professional associations, duty-bearers, as well as employees within key sectors on rights and service standards S4.1c. Promote the institutionalization of rights-based approaches and HIV-related issues into pre-service and in-service curricula S4.1.d Train service providers (employers, frontline staff, faith-based organizations) on rights-based approaches to reduce HIV-related stigma and discrimination S4.1.e. Mainstream the Positive Health Dignity Prevention (PHDP) framework into the national HIV response S4.1.f. Integrate HIV/AIDS policies and programmes in workplace wellness framework S4.1.g. Advance evidence-based data to inform interventions around HIV-related discrimination, gaps in the legal environment and gender issues S4.1.h. Engage faith-based organizations to lead efforts to reduce stigma and discrimination S4.1.i review and update the National Workplace Policy on HIV/AIDS S4.1.j Review, update and implement the Policy Management of HIV/AIDS in Schools	• Media campaign developed to address S&D • Modules developed for sensitization sessions with professional associations, duty bearers, as well as employees within key sectors • Members of professional associations, duty-bearers, as well as personnel from key sectors sensitized • Service providers trained • PHDP mainstreamed • Workplace wellness framework includes HIV/AIDS • Advocacy strategy and action plan developed • Advocacy technical documents/ briefs developed • Faith-based organizations sensitized • Updated National Workplace Policy on HIV/AIDS • Updated Policy Management of HIV/AIDS in Schools

Strategic Objective	Strategic Activities	Outputs
	S 4.2 Community-Led Monitoring S4.2.a Scale-up community-based monitoring of rights violations in education, health, justice, workplaces and communities S4.2.b. Mobilise and empower communities to improve the awareness of their rights and quality standards in accessing services S4.2.c. Strengthen the existing reporting and redress mechanisms to increase the number of cases/complaints for which redress is effected	• Community actors trained in monitoring rights violations • Reports on rights violations and stigma, and discrimination incidents • Data Dashboard implemented
	S4.3 Increased access to justice S4.3.a. Sensitize law enforcement agents on PLHIV and key population related issues S4.3.b. Promote legal literacy and increase access to HIV-related legal services, such as legal information and referrals S.4.3.c. Increase awareness of alternative forms of dispute resolution to effect redress for HIV-related discrimination S.4.3.d. Promote the implementation of the Health in All Policies Approach S.4.3.e. Promote law reforms, regulations and guidelines that prohibit discrimination and support access to HIV prevention, treatment, care and support S.4.3.f. Training of relevant stakeholders in navigating the reporting and redress mechanisms	• Law enforcement agents trained • Legal literacy training conducted with communities • Community actors and groups trained in legal literacy • Legal practitioners trained and supported to provide legal advice to PLHIV and members of the key population who report discrimination • Advocacy briefs • Stakeholders trained in navigating the reporting and redress mechanisms
	S4.4 Reduce gender-based violence S4.4.a. Integrate modules on GBV and inequality in training/sensitization sessions S4.4b. Promote multi-sectoral partnerships/collaboration for the implementation of GBV interventions S4.4.c. Implement programmes to increase the educational and economic empowerment of women living with or vulnerable to HIV infection S4.4.d. Promote the reform of domestic violence laws S4.4.e. Build the capacity of law enforcement agents to address gender-based violence	• Module on GBV developed • Key stakeholders trained on GBV • Gender-sensitization and GBV training integrated into training/sensitization sessions for duty-bearers • Modules on microfinance and entrepreneurship incorporated into key and vulnerable populations • Technical document/ advocacy brief • Law enforcement agents trained in GBV

Strategic Objective	Strategic Activities	Outputs
	S4.5 Strengthen Community Systems S4.5.a. Strengthen networks, linkages, coalitions and partnerships with communities to avoid duplication of interventions S4.5.b. Foster full partnership between community actors and national systems especially in the health, education, justice and social sectors	- Partners sensitized - Implementation of interventions monitored - Community actors engaged in national systems
	S4.6 Address social protection barriers to vulnerable and key populations access to services S4.6.a Improve access to social protection services for PLHIV, key and vulnerable population S4.6.b Promote for social protection policies and programmes that are concordant and HIV sensitive S4.6.c Develop the Social Protection Framework for health including HIV S4.6.d Incorporate the needs of people living with HIV into the disaster risk management plans	- Policy dialogue for the adoption of the Referral protocol MOU for use among and between RHAs, CSOs, FBOs & MDAs - Key and vulnerable population sensitized on social protection - Social Protection Framework for Health - Technical document/policy briefs

Strategic Objective 4 - Framework

Strategic Activities	Time Frame								Lead Agency (ies)
	Y1	Y2	Y3	Y4	Y5	Y6	Y7	Y8	
SO. 4 To promote respect for the rights of all persons in relation to HIV and AIDS issues in community, policy, legislation and programmes									
4.1 Reduce stigma & discrimination									
Develop and place media campaigns to address stigma and discrimination	X	X		X	X		X	X	MOHW
Sensitize professional associations and duty-bearers, as well as employees within key sectors on rights and service standards			X	X	X	X	X	X	MOHW
Promote the institutionalization of rights-based and HIV-related issues into pre-service and in-service curricula		X	X	X	X	X	X	X	MNS MOHW MOEY MOJ MLSS
Train service providers, employers, frontline staff, faith-based organization on rights-based approaches to reduce HIV -related stigma and discrimination	X	X		X		X		X	MOHW
Mainstreaming of PHDP framework into the national HIV response	X	X	X	X	X	X	X	X	MOHW
Integrate HIV/AIDS policies and programmes in workplace wellness framework	X	X	X	X					MLSS
Advance evidence-based data to inform interventions around HIV-related discrimination, gaps in the legal environment and gender issues		X			X			X	MOHW
Engage faith-based organizations to lead efforts to reduce stigma and discrimination	X	X	X	X	X	X	X	X	MOHW
Review and update the National Workplace Policy on HIV/AIDS		X	X	X	X				MLSS
Review, update and implement the Policy Management of HIV/AIDS in Schools	X	X	X					X	MOEYI
4.2 Community-Led Monitoring									
Scale-up community-based monitoring of rights violations in education, health, justice, workplaces and communities	X	X	X	X	X	X	X	X	MOHW MOEY MOJ MLSS MNS
Mobilize and empower communities to improve the awareness of their rights and quality standards in accessing services	X	X	X	X	X	X	X	X	MOHW MOEY MOJ MLSS MNS
Strengthen the existing reporting and redress mechanisms to increase the number of cases/ complaints for which redress is effected	X	X	X	X	X	X	X	X	MOHW MOEY MOJ MLSS MNS

Strategic Activities	Time Frame								Lead Agency (ies)
	Y1	Y2	Y3	Y4	Y5	Y6	Y7	Y8	
4.3 Increase access to justice									
Sensitize law enforcement agents on PLHIV and key and vulnerable populations related issues		X	X	X	X	X	X	X	MNS
Promote legal literacy and increase access to HIV related legal services, such as information and referrals			X	X	X	X	X	X	MLCA
Increase awareness of alternative forms of dispute resolution to effect redress for HIV-related discrimination		X	X	X	X	X	X	X	MOJ
Promote the implementation of the Health in All Policies Approach		X	X	X	X	X	X	X	MOHW
Promote law reforms, regulations and guidelines that prohibit discrimination and support access to HIV prevention, treatment, care and support	X	X	X	X	X	X	X	X	MOHW MLCA
Training of relevant stakeholders in navigating the reporting and redress mechanisms		X	X	X	X	X	X	X	MOHW MLSS MNS MOJ
4.4 Reduce gender-based violence									
Integrate modules on GBV and inequality in training/ sensitization sessions	X	X	X	X	X	X	X	X	BGA MOHW
Promote multi-sectoral partnerships/collaboration for the implementation of GBV interventions		X	X	X	X	X	X	X	BGA
Implement programmes to increase the educational and economic empowerment of women living with or vulnerable to HIV infection			X	X	X	X	X	X	BGA
Promote the reform of domestic violence laws			X	X	X	X	X	X	BGA
Build the capacity of law enforcement agents to address gender-based violence		X	X	X	X	X	X	X	BGA
4.5 Strengthen community systems									
Strengthen networks, linkages, coalitions and partnerships with communities to avoid duplication of interventions		X	X	X	X	X	X	X	MOHW
Foster full partnership between community actors and national systems, especially in the health, education, justice and social sectors	X	X	X	X	X	X	X	X	MOHW
4.6 Address social protection barriers to vulnerable and key populations' access to services									
Improve access to social protection services for PLHIV, key and vulnerable populations		X	X	X	X	X	X	X	MOHW MLSS MLGRD
Promote social protection policies and programmes that are concordant and HIV sensitive			X	X	X	X	X	X	PIOJ
Develop a social protection framework in health, including HIV			X	X	X	X	X	X	MOHW
Incorporate the needs of people living with HIV into the disaster risk management plans			X	X	X	X	X	X	MOHW

SO5 - TO DEVELOP AND MAINTAIN A ROBUST STRATEGIC INFORMATION FRAMEWORK THAT CAN GUIDE ALL ASPECTS OF THE HIV RESPONSE

The organization of data management within the MOHW is inadequately linked. The MOHW surveillance should be more cohesive to ensure optimal efficiency. The Epidemiology Units of the Regional Health Authorities are tasked with coordinating monitoring and evaluation activities for HIV and other diseases. All the posts in the RHA Epidemiology Unit are government of Jamaica posts, but there are no dedicated M&E posts (including for HIV). This has affected data management at the RHA level and the national level. CSOs and other partners in the response also have limited capacity for data management. Consequently, strategic information to direct the response must be improved.

Improvements have been seen in areas such as the HIV databases, routine monitoring and data quality. Even though these are evident, a holistic review was done using the 12-component assessment tool designed by UNAIDS, a universally accepted measurement tool of M&E systems. This assessment identified gaps in the current structure, which need addressing to ensure a robust strategic information system that guides the national programme. Gaps were identified in all 12 components, but strategies will be designed around the major issues affecting programme management at both the national and subnational levels.

The HIV response has established, to varying degrees, the 12 components or building blocks of a fully functioning HIV M&E system (UNAIDS). More specifically:

- 1.** Organizational Structure for M&E: The HSTU oversees the implementation of the National M&E plan, including data collection, quality assurance, analysis and dissemination. M&E functions occur at various levels among stakeholders.
- 2.** Human capacity for M&E: Capacity building in priority areas, including M&E, has been a strategic priority in the NSP since 2007. This includes M&E training workshops, mentorship programs, technical assistance and advanced training on evaluation methodologies.
- 3.** M&E partnerships: Stakeholders are encouraged to contribute to the M&E system and to recognize their role in the achievement of targets. Until recently, the Jamaica Monitoring and Evaluation Reference Group (MERG) routinely provided the space for the partnership with stakeholders, where HIV related research was shared that informs future NSPs.
- 4.** M&E Plan: M&E plans have been developed and costed. These are aligned with the National Strategic Plan.
- 5.** Costed M&E work plan.
- 6.** M&E advocacy: Stakeholders endorsed the M&E plan and participated in training, dissemination and data utilization exercises. M&E roles have been assigned to relevant staff.
- 7.** Routine Program Monitoring: A combination of electronic databases and paper-based systems captures important data on stakeholder activities. Reports from these are coordinated at all levels of the system.
- 8.** Surveys and Surveillance: Case based reporting of persons diagnosed with HIV is a core part of Jamaica's HIV surveillance system and occurs both actively and passively. This system provides information on the burden of disease, trends in HIV diagnosis and deaths, as well as risk factors. Additionally, surveys of the general and key populations, such as MSM/TG, homeless, prisoners, and SW, provide prevalence data as well as data on risk behaviours and access to HIV services. These surveys are predominantly donor-funded. Operational research needs continue to be identified by stakeholders and form part of the research agenda.
- 9.** M&E database: Some databases have been established to capture persons reported with HIV, PLHIV in care, STI data and routine monitoring data and outreach testing.
- 10.** Supervision and data auditing: Audits of databases and data collection tools have been conducted with a view to improving the efficiency and accuracy of the strategic information system.

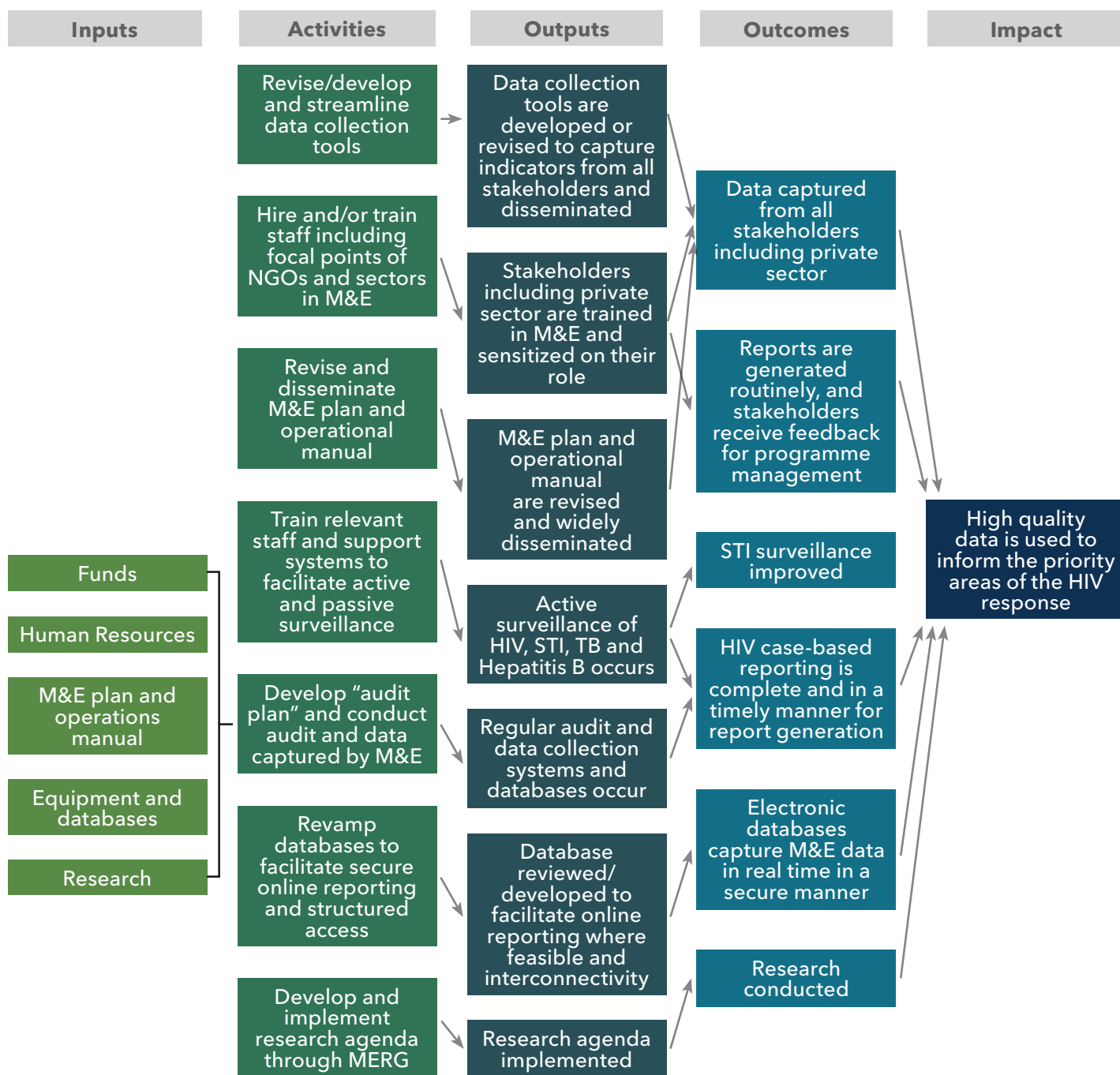
- 11.** Evaluation and research: various stakeholders conduct Research and evaluation of the NSP: these include behavioural surveys, surveillance of general and key populations.
- 12.** Data dissemination and use: Regular meetings of stakeholders are held at various levels, including a comprehensive annual review, to review indicators and monitoring data, which guide priority areas. Other routes of dissemination include presentations at scientific conferences, bulletins and publications in peer-reviewed scientific journals.

Strategies will address the following gaps identified in the assessment:

- Absence of supportive supervision, infrequent audits and poor data quality
- Inadequate data use at all levels
- Standalone HIV databases
- Poor communication of M&E related material
- No standardized organizational structure around M&E and absence of dedicated M&E role
- Lack of a research agenda and work plan to guide research and programme evaluation
- Inconsistent data collection

Logic Model for Strategic Information

Problem Statement: Timely reporting, gaps in surveillance and limited research prevent accurate monitoring and evaluation of the HIV response



Strategies

- **Improve the Monitoring & Evaluation organizational structure and capacity**
 - Review and Evaluate M&E Organizational structure
 - Implement M&E training manual
 - Update M&E roles and responsibilities
 - Adequate M&E staff at all levels of the programme
- **Improve M&E supportive supervision and Data quality**
 - Implement supportive supervision protocol
 - Routine data quality activities
- **Improve HIV data management**
 - Integrate HIV management systems
 - Link adjunct systems
 - Update data sharing policy
- **Increase early communication and data use**
 - Implement M&E communication plan
 - M&E publications
 - Data review processes
- **Improve research and evaluation**
 - Re-establish the MERG
 - Develop the research agenda
 - Programme evaluation

Strategic Objective	Strategic Activities	Outputs
SO5 To develop and maintain a robust monitoring and evaluation framework that can guide all aspects of the HIV epidemic	S5.1 Improve organizational structure and human capacity. Review and Evaluate the Human Capacity Development Plan for M&E	• Implementation of the M&E training manual and SOP. • M&E organizational structure evaluated in the third year. • Job descriptions updated, including M&E roles and responsibilities. • Dedicated M&E staff for all levels of the national programme. • National database of trainees, trainers and technical service providers.
	S5.2 Improve supportive supervision, data audits and data quality Create an audit framework for technical and administrative processes Establish a data auditing team at the national level. Conduct biannual audits of sites and the private sector.	Revision and implementation of the supportive supervision protocol and framework. • Implementation of data quality protocol • Biannual audits conducted
	S5.3 Increase data use at all levels Dedicated M&E Staff for data analysis and interpretation with adequate supervision and accountability Encourage the development of parish data bulletins for dissemination nationally	• Reports are generated routinely for feedback and programmatic decisions • M&E cadre of staff increased to 75% of the proposed M&E HR framework by 2025 • Monthly subnational data review meetings • Quarterly national data review meetings • Quarterly Parish Bulletins
	S5.4 Improve HIV data management and sharing Integration of HIV data management systems. Collaboration with Pharmacy and Laboratory databases Develop a data sharing agreement with MOHW and other MDAs that ensures the timely sharing of HIV relevant data with the national programme.	• Biannual Epidemic Tracking Report • Improved PLHIV monitoring by subnational treatment facilities • Data sharing agreement developed

Strategic Objective	Strategic Activities	Outputs
	S5.5 Increase communication and culture around M&E	Implementation of the communication plan
	S5.6 Improve data collection and monitoring	- Updated M&E Operational Guidelines. - Regional M&E Plans developed - Standardized data collection tools.
	S5.7 Improved research and evaluations	- Recommissioned MERG - Research Agenda Developed and implemented - Evaluation of implemented strategies

Strategic Objective 5 - Framework

Strategic Activities	Time Frame								Lead Agency (ies)
	Y1	Y2	Y3	Y4	Y5	Y6	Y7	Y8	
SO5 - To develop and maintain a robust monitoring and evaluation framework that can guide all aspects of the HIV epidemic									
5.1 Improve organizational structure and human capacity. Develop a Human Capacity Development Plan for M&E									
Evaluate the M&E structure and human capacity development plan			X					X	MOHW
Implement M&E training manual and SOP.	X	X	X	X	X	X	X	X	MOHW
Update job descriptions, including M&E roles and responsibilities.			X					X	MOHW
National database of trainees, trainers and technical service providers.	X	X	X	X	X	X	X	X	MOHW
5.2 Improve supportive supervision, data audits and data quality									
Conduct supportive supervision activities	X	X	X	X	X	X	X	X	MOHW
Revise audit tool for data quality, management & analysis		X		X		X		X	
Conduct biannual audits of sites and the private sector	X		X		X		X		
5.3 Increase data use at all levels									
Establish M&E structure and employ 75% of the positions in the proposed & adopted M&E structure	X	X	X	X	X	X	X	X	MOHW
Parishes to develop quarterly bulletins as a tracking and monitoring tool	X	X	X	X	X	X	X	X	MOHW RHAs
5.4 Improve HIV data management and sharing									

Strategic Activities	Time Frame								Lead Agency (ies)
	Y1	Y2	Y3	Y4	Y5	Y6	Y7	Y8	
Merge the prevention data with TSIS	X								MOHW
Develop a data sharing agreement that ensures timely sharing of HIV relevant data with the national programme, with MOHW and its executive agencies.	X			X					MOHW
5.5 Increase communication and culture around M&E									
Implementation of the communication plan	X	X	X	X	X	X	X	X	MOHW
5.6 Improve data collection and monitoring									
Update the National plan and develop a regional M&E Plan	X				X				MOHW
Update M&E Operational Guidelines.	X				X				
Revise and standardize data collection tools.	X	X	X	X	X	X	X	X	MOHW
Train staff to facilitate active and passive surveillance to strengthen case-based reporting		X			X				MOHW
5.7 Improve research and evaluations									
Recommission J-MERG	X								MOHW
Develop and implement the Research Agenda		X	X	X	X	X	X	X	MOHW
Evaluation of Implemented strategies				X			X		MOHW

LIMITATIONS

Lack of an evaluation to assess gaps in implementation

A key limitation to guide the development of strategies in the current NSP has been the lack of an evaluation for the previous National Integrated HIV Strategic Plan (2014-2019). While a medium-term evaluation was conducted, a final evaluation was not carried out.

Availability of data and data quality

Ongoing activities to improve the data quality of the treatment database and surveillance system found that there were many duplicated records, unreported risk factor data, and unreported viral suppression outcomes. Surveillance data is also limited and not timely. Therefore, the UNAIDS Spectrum model estimates are used, but the country recognizes the limitations of these models. Population size estimates

for key populations such as transgender persons are possibly underestimated within the Jamaican context, which may result in inflated prevalence ratios. The private sector's contribution in this area is also limited, and this affects the quality of data on coverage of programs, especially pertaining to the treatment of PLHIV. Therefore, key data to design evidence-based policies is limited. Efforts to address data availability and quality will continue, and the discrepancies between the surveillance and treatment databases need to be urgently resolved.

Data limitations also prevent key strategies from being included, such as those targeting transgender persons. The NSP aims to create evidence-based strategies, but a lack of data on specific populations, despite the significantly high prevalence identified, limits the design of interventions.

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